



Situation Analysis

SUDAN CRISIS

Period: 08/06/26 – 14/06/26

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Introduction

Purpose

This report provides a comprehensive overview of the current humanitarian situation, focusing on both operational environment factors and sectoral analysis. It aims to support informed decision-making for humanitarian partners and stakeholders.

About GANNET

GANNET is a Generative AI Initiative from Data Friendly Space, offering AI-powered tools built to make knowledge and real-time information accessible to our community in a resource-efficient and ethical way. It provides tools such as the GANNET Virtual Assistant and GANNET SituationHub to help humanitarian actors access, analyze, and utilize critical information for their operations.

GANNET SituationHub

The GANNET SituationHub is a generative AI-powered digital public good designed to identify and analyze humanitarian data from thousands of trusted sources in real time. By transforming complex data into clear insights, it enables faster, more effective response strategies and improved coordination among stakeholders.

GANNET Virtual Assistant

The GANNET Virtual Assistant is a verified chatbot designed for humanitarian quick decision-making. Built on cutting-edge Generative AI and trained with humanitarian expert-reviewed data, it provides real-time information. The GANNET Virtual Assistant facilitates rapid data access and synthesis for crisis response.

Methodology Overview

The Situation Hub's analytical process uses a structured Human-in-the-Loop workflow that combines automated analysis retrieval through GANNET Virtual Assistant, DFS's AI-powered humanitarian assistant, with expert review by the DFS analysis team. GANNET automates the extraction and generation of crisis-related insights based on structured prompts, which are aligned with a tailored analytical framework. These outputs are then reviewed, validated, and refined by analysts to ensure accuracy, contextual relevance, and operational utility. The workflow is organized into four key phases: planning and setup, ongoing operations, quality assurance, and system evolution, ensuring a balance between speed and analytical rigor.

Planning, Setup & Quality Control

In the planning phase, analysts develop a contextualized analytical framework, largely based on the Joint Intersectoral Analysis Framework (JIAF), and convert it into structured prompts for AI-assisted data extraction. These prompts are refined over time to improve output quality. Data sources are vetted for reliability, and translations are reviewed for accuracy. Using a centralized Content Management System (CMS), analysts review and curate AI-generated outputs, validate them against external sources, and make refinements to enhance clarity and relevance. The CMS enables collaborative editing and version tracking, ensuring outputs align with humanitarian needs.

Bias Mitigation

To minimize bias, the team diversifies data sources, cross-verifies information, and continuously evaluates the AI's performance. Analysts are crucial in identifying and correcting inaccuracies, especially in automated categorization. Future improvements aim to enhance source credibility checks and sentiment analysis, further strengthening the reliability of the Situation Hub's outputs.

Crisis Overview

The SituationHub landing pages use a standardized yet customizable template to deliver consistent, comprehensive crisis overviews. These pages integrate static content with dynamic data from sources like ACLED and DTM via open APIs, enabling interactive visuals with real-time updates. Regular reviews and verification with in-country partners ensure accuracy, while manual updates are used for sources not connected to APIs.

Media Monitoring

The media monitoring process has evolved from a manual, list-based approach to an AI-assisted system integrated into the CMS. This allows analysts to efficiently review, filter, and contextualize media narratives, while automation helps track emerging trends.

Overview

The following overview has been generated using the information available up to June 26, 2026. It provides a synthesized summary and key insights into the crisis based on the most recent data accessible at that time.

Summary

Sudan is facing one of the world's largest humanitarian and displacement crises as the conflict between the Sudanese Armed Forces (SAF) and the Rapid Support Forces (RSF), which began in April 2023, enters its fourth year in 2026. The conflict has triggered widespread civilian suffering, economic collapse, mass displacement, and severe regional instability, with humanitarian conditions continuing to deteriorate across much of the country.

An estimated 19.5 million people are currently experiencing acute food insecurity, making Sudan the world's largest hunger crisis. Famine conditions have been confirmed in el-Fasher (North Darfur) and Kadugli (South Kordofan), while multiple areas across Darfur and Kordofan remain at high risk of famine amid ongoing conflict, siege tactics, market disruption, and severe restrictions on humanitarian access. Children continue to bear a disproportionate burden of the crisis, with millions exposed to acute malnutrition, disease outbreaks, and heightened mortality risks.

Fatality estimates remain highly contested due to limited access, communication blackouts, and verification constraints. The Armed Conflict Location & Event Data Project (ACLED) recorded nearly 30,000 reported deaths by late 2024, while independent analyses and international media estimates suggest the true death toll may exceed 150,000 people. Civilians continue to face widespread violence, including indiscriminate shelling, aerial bombardments, drone attacks, and ethnically targeted violence, particularly in Darfur and parts of Kordofan.

Sudan's health system remains severely degraded, especially in active conflict zones where many facilities are non-operational or functioning only partially. Although the nationwide cholera outbreak that spread across all 18 states between 2024 and early 2026 was declared contained in March 2026, underlying conditions—including mass displacement, damaged health infrastructure, and poor sanitation—continue to create a high risk of renewed outbreaks and other communicable diseases.

Displacement continues at an unprecedented scale. More than 14 million people have been displaced since the start of the conflict, including approximately 9 million internally displaced persons (IDPs) and over 4 million refugees in neighboring countries, making Sudan the world's largest displacement crisis. At the same time, limited and fragile returns to parts of Khartoum and Al Jazirah have been reported since early 2026 following shifting frontlines, though returnees face widespread destruction, insecurity, and lack of basic services and livelihoods.

Militarily, the conflict has continued to fragment since late 2025, with intensified fighting in Darfur and Kordofan driving further civilian displacement and humanitarian access constraints. The RSF maintains significant influence across much of Darfur and continues offensive operations in Kordofan, while the SAF retains control over key eastern and northern urban centers. The growing use of drones, aerial strikes, and long-range attacks has increasingly impacted civilian infrastructure, including hospitals, schools, markets, and displacement sites, further compounding humanitarian needs.

Ethnic violence and sexual and gender-based violence (SGBV) continue to escalate, particularly in Darfur. Human rights organizations and monitoring networks have documented widespread abuses, including conflict-related sexual violence, forced displacement, arbitrary detention, and targeted attacks on ethnic communities. Despite sustained international pressure, including sanctions imposed by the United States and United Kingdom on RSF leaders and affiliated networks, diplomatic and mediation efforts have yet to achieve a durable ceasefire or political settlement.

The conflict is also generating increasing regional spillover, with large refugee flows, cross-border insecurity, and growing pressure on already overstretched humanitarian systems in neighboring countries. Humanitarian operations inside Sudan remain critically underfunded, with the World Food Programme warning in 2026 that severe funding gaps are threatening emergency food pipelines, ration levels, and the continuity of lifesaving assistance.

As of mid-2026, Sudan remains one of the world's most severe and complex humanitarian emergencies, characterized by prolonged armed conflict, fragmented territorial control, mass displacement, and shrinking humanitarian space. Without urgent improvements in humanitarian access, sustained financing, and renewed diplomatic engagement, the crisis is likely to deepen further, with serious implications for civilian populations and broader regional stability.

Key Insights

1. Sudan's conflict between the Sudanese Armed Forces (SAF) and the Rapid Support Forces (RSF) has created the world's largest displacement crisis. Since the conflict began in April 2023, more than 14 million people have been forcibly displaced, including approximately 9 million internally displaced persons (IDPs) across all 18 states and more than 4 million refugees who have fled to neighboring countries including Chad, South Sudan, Egypt, Ethiopia, Libya, and the Central African Republic. The International Organization for Migration (IOM) continues to describe Sudan as the world's largest internal displacement crisis, with displacement patterns remaining highly fluid as fighting intensifies across Kordofan and violence persists in Darfur. Continued insecurity, deteriorating humanitarian conditions, and repeated attacks on civilian areas are driving ongoing population movements both within Sudan and across regional borders.

2. A catastrophic humanitarian situation persists in Sudan, with nearly 25 million people—more than half the population—experiencing acute food insecurity. Famine conditions have been confirmed in el-Fasher (North Darfur) and Kadugli (South Kordofan), with at least 20 additional districts across Darfur and Kordofan at serious risk of sliding into famine. Global acute malnutrition exceeds 15% in several areas and surpasses 30% in parts of Darfur and Kordofan—above IPC Phase 5 famine thresholds.

An estimated 825,000 children are projected to suffer severe wasting in 2026. Simultaneously, the health system has effectively collapsed: 70% of facilities are non-functional, and cholera has spread to all 18 states with over 113,000 cases and 3,000 deaths since 2024.

3. The conflict has expanded significantly in December 2025. The RSF has consolidated control over most of Darfur following el-Fasher's fall in late October and is now advancing east into Kordofan. In early December, the RSF reportedly seized the SAF's 22nd Division base in Babanusa and captured the strategic Heglig oil field on December 8, halting oil production and threatening state revenues. Cities including Kadugli and Dilling are now besieged. The SAF retains control of the north, east, and center—including Khartoum and parts of Kordofan—but faces sustained pressure as the RSF mobilizes to seize further territory and strategic resources.

4. Sudan is facing multiple disease outbreaks across all 18 states, with cholera, malaria, and dengue widespread. Over 113,000 cholera cases and 3,000 deaths have been recorded since 2024, with outbreaks concentrated in overcrowded displacement sites where water and sanitation systems have collapsed. Malaria has reached epidemic proportions, with Sudan accounting for 41% of global cases. The collapse of water supply systems and sanitation services continues to worsen the public health crisis.

5. Humanitarian operations face severe challenges as access constraints and funding shortfalls intensify. WFP has warned of imminent pipeline breaks and plans to cut rations from January 2026—reducing to 70% in famine areas and 50% elsewhere—with full pipeline breaks possible by April 2026 without an urgent \$695 million injection. The 2025 HNRP requiring \$4.2 billion remains only about 25% funded. Despite these constraints, humanitarian actors reached 1.6 million people in Khartoum between January–June 2025 and continue life-saving operations, while approximately 2.6 million returnees have been recorded across stabilized areas.

6. Civilian harm has escalated dramatically in December 2025. A devastating drone strike on a kindergarten and hospital in Kalogi, South Kordofan, on December 4 killed at least 114 people, including 63 children. A separate drone attack killed six UN peacekeepers. The UN Human Rights Office documented at least 269 civilian deaths in Kordofan since October 25. WHO has recorded 63 attacks on healthcare in Sudan in 2025, resulting in 1,611 deaths and 259 injuries. Sieges and starvation tactics are being used as weapons of war, with an estimated 70,000–100,000 people trapped in el-Fasher under famine conditions.

7. Systematic sexual and gender-based violence continues at alarming levels. The Strategic Initiative for Women in the Horn of Africa (SIHA) verified 1,294 SGBV incidents across 14 states since April 2023, finding that rape accounted for more than three-quarters of incidents, 225 cases involved children as young as four, and where perpetrators were identified, 87% were attributed to RSF. Patterns include initial home invasions with rape, public-space attacks, and long-term detention with gang rape, torture, and forced marriage—with victims from non-Arab communities including Masalit, Fur, and Zaghawa directly targeted. These violations have been characterized as systematic, warranting investigation for war crimes and crimes against humanity.

8. International pressure on conflict parties has increased. In December 2025, the US imposed sanctions on a transnational network recruiting Colombian mercenaries for the RSF, while the UK sanctioned senior RSF commanders and pledged £21 million in emergency assistance. A US-led Quartet (US, Saudi Arabia, UAE, Egypt) has proposed a three-month humanitarian truce followed by a permanent ceasefire and civilian transition, though the SAF rejected the proposal while RSF declared a unilateral ceasefire that has not held on the ground. UN-led Geneva talks are planned, but prospects for a negotiated settlement remain dim amid continued fighting and external support to warring parties.

9. Sudan is experiencing rapid fragmentation of state authority, with parallel and competing governance structures emerging across SAF- and RSF-controlled territories. While the SAF retains formal control over parts of Khartoum, the north, and eastern Sudan, and the RSF exercises de facto authority across most of Darfur and expanding areas of Kordofan, national-level governance has largely broken down. Public administration, taxation systems, justice mechanisms, and service delivery have been severely disrupted or replaced by localized and often militarized governance structures. This fragmentation has weakened national coordination of humanitarian response, complicated negotiations over access, and deepened regional disparities in protection and service provision. As a result, civilians are increasingly governed through fragmented, militarized authorities rather than a unified state system, accelerating institutional collapse alongside the ongoing conflict.

10. Sudan's conflict has entered a deeply fragmented phase marked by the collapse of centralized state authority and the emergence of competing SAF- and RSF-controlled governance systems, leaving civilians increasingly governed by localized and militarized structures. This breakdown has severely disrupted basic state functions—including service delivery, justice, and taxation—while undermining coordinated humanitarian access and response. As a result, protection risks, access constraints, and regional disparities in humanitarian conditions are intensifying, accelerating both institutional collapse and the scale and severity of the humanitarian crisis as of mid-2026.

Economy

Current economic situation

Sudan's macroeconomy has collapsed into a severe crisis, with the Sudanese pound trading at record lows on the parallel market (around 4,400 SDG per USD) amid wartime economic collapse and rampant black-market foreign exchange activity [10][5]. Authorities have pursued emergency measures — including Central Bank and government efforts to boost gold production and exports and emergency fiscal measures — to stabilise the pound, even as core financial services, broker networks and remittance flows have collapsed and trade and transport disruptions persist [5][6][10]. The government reports a 73% poverty rate (including diaspora) and has announced a 2026–2030 five-year livelihoods plan alongside claims of partial service restoration (e.g., electricity restored to up to 80% of Khartoum), while acknowledging ministries are operating far below capacity [9]. FEWS NET and related humanitarian analyses warn that Emergency (IPC Phase 4) outcomes are expanding across Greater Darfur, Greater Kordofan and Blue Nile, and that two in five people in Sudan face crisis or worse levels of food insecurity, with nearly 200,000 in catastrophic (IPC Phase 5) conditions in some areas [6][41][11]. Market assessments show markets remain functioning in many localities, but affordability — driven by loss of purchasing power — is the binding constraint, with cash-based assistance recommended where markets operate and continued market fragility noted across monitored sites [42][6].

Impact of the conflict on the economy

The conflict has severely distorted trade routes, market cycles and economic output. Competing territorial control — SAF holding Port Sudan and central export infrastructure while RSF controls much of Darfur and Kordofan's gold and livestock areas — has fragmented export chains, incentivised smuggling (notably gold flows to the UAE) and shrunk legitimate foreign-exchange earnings [27][14][31]. Gold revenues and illicit mineral flows have materially financed the war economy: the RSF produced an estimated 10 tonnes of gold in 2024 (approximately US\$860 million) while gold prices rose around 150% since 2024, briefly breaking US\$5,000/oz in March 2026 [14][7].

Food and fuel inflation spiked sharply in April 2026: sorghum rose +23% month-on-month and over 170% above the five-year average, wheat +17.5% month-on-month and approximately 187% above average, millet +14% and approximately 134% above average, while diesel rose 55% month-on-month on the official market and fuel costs increased 83–144% year-on-year — raising transport costs and compressing household purchasing power [6][11]. April 2026 market monitoring found over half of localities had generally functional markets, but almost two-thirds scored below average in affordability and resilience, with Dilling, As-Sunut, Nyala Shimal and Kereneik performing poorly across availability, access and infrastructure indicators [6].

Targeted strikes and infrastructure damage have severed key trade and aid corridors — most notably the Ardamata bridge in El Geneina (damaged in early June 2026) and bridges on the Kadugli–Dilling road — reducing commercial traffic and constraining humanitarian access ahead of the rainy season [5][8][1]. Regional shocks linked to Strait of Hormuz disruptions have fractured humanitarian and commercial logistics, raising freight and insurance costs, delaying critical consignments to Sudan (including a two-month delay of medical supplies) and inflating operational costs as of early June 2026 [18][41][55]. Cross-border population and return movements also reshape market cycles: DTM recorded 766,910 people crossing back into Sudan between January 2024 and April 2026 (82% from Egypt), affecting labour, remittances and local demand patterns [20]. Together, currency depreciation, record food and fuel inflation, disrupted routes and port operations, and war-financing via illicit exports are increasing the risk of market collapse and famine in parts of North Darfur and South Kordofan during the 2026 lean season [6][14][11].

Stability of the currency and banking systems

The conflict has severely disrupted financial infrastructure, producing high inflation, currency depreciation, liquidity shortages, fragmentation of the financial system and the collapse of formal banking in nearly half of the country [24][33]. The Sudanese pound plunged on the black market to about 4,400 per US dollar; airlines began requiring ticket purchases in US dollars and traders rely on black-market foreign exchange, while the Egyptian pound traded at around 87 Sudanese pounds — all signalling a sharp loss of confidence in the national currency and formal foreign-exchange channels [5][10]. Authorities in the army-controlled government, together with the Central Bank, have pursued emergency measures to boost gold production and exports to artificially stabilise the pound, measures that analysts describe as stopgap and insufficient to address core problems [5]. A large rise in global gold prices (approximately +150% since 2024, briefly above US\$5,000/oz in March 2026) has increased revenue flows tied to mining, strengthening incentives for armed groups to control mining and transit routes and complicating monetary stabilisation efforts [14][27]. The Finance Minister chaired a June 8, 2026 meeting with the African Development Bank, WFP and other partners to secure urgent financing for the 2026–27 agricultural season and to follow up on the Port Sudan water project, indicating continued engagement with international financial institutions to support food-security financing despite macroeconomic instability [30].

Employment

Sudan's labour market has been severely disrupted by the conflict, with high inflation, currency depreciation, liquidity shortages and the collapse of formal banking in nearly half the country undermining formal employment and market functioning [33][5]. Poverty is reported at 73% (covering Sudanese inside and outside the country), over 11 million people are internally displaced, and millions more have lost income-earning opportunities — driving heavy reliance on cash and voucher assistance, with over 90% of affected households preferring cash, while local mutual-aid networks and the Collective Sudan platform deliver coordinated CVA [9][33][24].

The Ministry of Human Resources announced a five-year livelihoods plan (2026–2030) to support over three million small-scale projects, prioritising in 2026 the implementation of 500,000 projects for youth and women and agricultural inputs programmes, backed by partnerships and resumed financing windows via banks, the Zakat Chamber and the Anti-Poverty Commission [9]. Minister Mua'tassim Ahmed Salih stated the ministry is operating at roughly 10% capacity, that over 10,000 projects are ready for beneficiaries,

and that transport and other measures are being mobilised to facilitate voluntary returns and labour reintegration [9].

Market and value-chain assessments in April–May 2026 found markets largely functional at the state level but with low affordability and resilience, with supply-chain disruptions and the loss of financial services constraining farmers, traders and informal employment opportunities [6][33]. Authorities have pursued emergency measures — including boosting gold production and exports and an emergency 2026 budget with very high tax assumptions — but analysts warn these stopgap policies are unlikely to restore sustainable employment without improved security and the reopening of trade and financial channels [5].

Politics

Overall political landscape

The conflict has produced de facto parallel administrations: the Sudanese Armed Forces (SAF) re-established a government in Port Sudan (including the appointment of Kamil Idris as prime minister) while the Rapid Support Forces (RSF) created a separate administration centered in Nyala under Mohamed Hamdan "Hemedti," consolidating rival control over different regions [132][92]. Major international and regional actors have coalesced around a civilian-led, time-bound dialogue and roadmap calling for an inclusive Sudanese civilian transition, with explicit support for broad participation of women, youth, and civil society, and warnings of measures against those who undermine it [119][114][111]. The Nairobi roadmap and related processes announced in May–June 2026 seek a renewable, internationally monitored ceasefire and a political track that notably debates excluding leaders of the Islamist movement from political participation — a point identified as a core obstacle to a comprehensive settlement [94]. On the ground, the RSF tightened security and redeployed forces across Darfur and Kordofan following the defections of senior field commanders in early June 2026, withdrawing units from some frontlines and reinforcing others from Nyala and El Fasher [133]. Local governance remains partly functional in some areas: in West Kordofan's Ghubaysh, local command structures, ministries, and administrative authorities continued to operate and facilitated access for responders despite heavy militarization and looting of health infrastructure earlier in May–June 2026 [159]. The rule of law is severely weakened by entrenched "deep state" networks and a politicized judiciary that have historically undermined judicial independence and accountability [101]. Persistent calls for accountability continue from rights groups and victims: Human Rights Watch and other actors have urged investigations into alleged abuses by commanders who switched sides, and victims have filed war-crimes complaints against RSF members in regional jurisdictions for atrocities dating from April 2023 through March 2025 [137][157].

Political stability

Western governments and regional and international organizations issued a joint statement after Quintet consultations in Addis Ababa (3–5 June 2026) calling for an urgent humanitarian truce, a permanent ceasefire, and an inclusive, civilian-led political process to restore democratic civilian rule; signatories included the United States, Belgium, France, Germany, Greece, Italy, Norway, the United Kingdom, the African Union, the EU, IGAD, the League of Arab States, and the UN [114][119][113]. Those international partners pledged support to complete a broad Sudanese dialogue within six months and warned that measures could be taken against individuals or groups that obstruct the civilian transition process [119][113]. The Samud final communiqué (7–9 June 2026) urged the SAF and RSF to accept a humanitarian truce, guarantee unconditional humanitarian access, release detainees, remove armed formations from major cities, open civic space, and begin a political process toward an independent civilian government with accountability and transitional justice measures [124]. Sudan's de facto political fragmentation — with parallel administrations and competing authorities across regions — continues to undermine checks and balances and complicate prospects for a unified civilian transition [132][93]. Attacks on key civilian infrastructure, notably the drone strike that damaged the Ardamata Bridge in El Geneina and forced suspension of traffic in mid-June, have been condemned by external actors and directly threaten humanitarian access and the viability of political confidence-building measures [98][87]. International pressure that conditions ceasefire and humanitarian access on a civilian-led transition increases external influence on domestic political competition, intensifying leverage on armed actors while also raising risks of politicized implementation of transition benchmarks [113][114].

Elections

Media reporting between 11–12 June 2026 indicated that the RSF-aligned "Ta'asis" (Founding) coalition organised parallel secondary school certificate exams in Darfur and parts of Kordofan, with around 10,000 students sitting tests in some 83 centres; female examinees made up about 74 percent of participants, and the events were publicly marked by Lieutenant General Mohamed Hamdan Dagalo and Prime Minister Mohamed Hassan al-Ta'aishi [147][140][118]. UNICEF on 10 June 2026 called for all Sudanese students to be allowed to sit the same nationally recognised examinations across conflict lines, warning that fragmented certification risks long-term inequalities and that education must remain neutral [127]. Observers and press warned that the parallel exams are unprecedented and risk deepening political and institutional divisions by demonstrating a non-state-aligned authority performing state-like functions and politicising education [140][147][118]. Together, these developments signal increased influence of a powerful non-state actor over public institutions and raise concerns about the erosion of neutral civic spaces that underpin fair access to state credentials and future opportunities [127][140][147].

Political stakeholders

On the ground, power is divided between the SAF under General Abdel Fattah al-Burhan, which operates from Port Sudan and retains central-state institutions, and the RSF under General Mohamed Hamdan "Hemedti" Dagalo, which controls much of Greater Darfur and has established a self-proclaimed civilian administration in Nyala that issues decrees, collects informal taxes, and prints identification documents [132][92]. Armed movements and Juba-signatory groups — most prominently the Sudan Liberation Movement (SLM) and the Justice and Equality Movement (JEM) — remain influential regional stakeholders; the SLM under Mona Arko Minawi fields an estimated 7,000–10,000 fighters, giving it significant leverage in Darfur [97][94]. The Nairobi roadmap (announced May 2026) proposes a three-track process combining humanitarian measures, an internationally monitored renewable ceasefire, and a political transition

that seeks to exclude both the SAF and the RSF from the political track and to bar leaders of the Islamist movement — including the former National Congress Party — from political participation [94]. In early June 2026, international and regional partners backed this civilian-led roadmap and tasked the Quintet to prepare an inclusive, time-bound Sudanese civilian dialogue intended to produce a roadmap to independent civilian rule [111][114][119][115]. A resilient "deep state" and patronage networks — described as interacting political, military, economic, institutional, community, and legal structures that control personnel, resources, and markets — continue to shape decision-making, protect vested interests, and impede rapid civilian reforms [101]. This mix of parallel armed administrations, entrenched patron-client structures, and disputed inclusion of Islamist networks complicates efforts to unify armed forces, negotiate accountability, and implement a civilian-led transition [94][93][101].

Historical barriers to power

Sudan's exclusionary access to political power is rooted in colonial-era centralization that prioritized urban centres over rural peripheries, producing long-standing structural imbalances between Khartoum and outlying regions [109][86]. Post-independence governance reproduced that centralizing pattern — formal decentralization often lacked real autonomy or resources — leaving Darfur, South Kordofan, Blue Nile, and eastern regions chronically under-represented and fueling recurrent armed grievances [86][93]. Repeated coups, the entrenchment of a "deep state," and the militarization of politics have blocked genuine civilian rule and institutionalized patronage networks that limit marginalized groups' meaningful participation [99][101][132]. The 2023 war hardened these patterns by producing rival de facto administrations and parallel governance structures, further fragmenting political access for communities outside centres of control [132][92]. Political initiatives since 2025–May 2026 have reflected contested approaches to inclusion — some proposals, such as the Nairobi roadmap, seek to bar former regime and certain armed or Islamist actors from post-conflict politics, underscoring an ongoing debate over exclusionary versus inclusive settlement terms [94]. Sudanese civic and political actors continued to call in early June 2026 for a national project that counters hate speech, counters efforts to revive dissolved regime networks, and expands participation for marginalized regions as a precondition for sustainable access to power [124].

Trust in institutions

Public trust in Sudanese state institutions is low and has been eroded by decades of exclusion, weakened institutions, and the prolonged war that began in April 2023 [100][132][97]. Trust in health authorities and health-related messaging has further declined, driven by attacks on healthcare, the emergence of parallel humanitarian service delivery, and polarising social media narratives that undermine perceived protection [169][109]. Political legitimacy is fragmented: Islamist networks retain influence within parts of the state and have broadly aligned with the SAF, while the RSF has established parallel administrative structures and allied coalitions, further reducing confidence in unified governance [94][132]. Key sources of organised support for governing authorities include international and regional backers pressing for an inclusive civilian transition and providing diplomatic leverage (including the US, EU, AU, IGAD, League of Arab States, and the UN), domestic military and paramilitary networks (including Islamist-linked cadres), and entrenched "deep state" institutional and economic patronage networks [115][114][94][101].

Security & Stability

Security overview

Combat and aerial/drone strikes have intensified across multiple fronts in June 2026, with repeated strikes on markets, villages, and civilian infrastructure causing significant civilian deaths and disrupting humanitarian access [236][202]. Active frontlines persisted in northwest North Darfur, Greater Kordofan, and Blue Nile in May–June 2026, with ground clashes and aerial strikes reported in Dilling, Kauda, Heiban, Kurmuk, Bau, and Geisan [202][229]. A drone strike on Ghubaysh market in May 2026 reportedly killed at least 28 civilians; Ghubaysh hosts large IDP populations (DTM 12 Apr 2026: ~22,153; local authorities: 68,000; SI assessment ~30,000) and has become a recurring humanitarian and security hotspot due to militarization and critically reduced water, food, and health services [221]. A reported arson campaign and intercommunal fighting in Kauda from 2–16 May caused at least 61 civilian deaths and burned the central market and surrounding settlements [265]. Drone attacks and aerial bombardment in May also targeted markets and trade routes, with strikes reported across Kordofan and Nyala, disrupting commerce and humanitarian access [202][190].

Renewed intercommunal violence between the Beni Halba and Salamat in South Darfur triggered deadly clashes and the displacement of over 13,000 people within a week (4–6 June 2026) [202][238][210]. High-profile defections in April–May 2026—including RSF and SPLM-N Agar commanders—have altered cohesion and local control dynamics, fragmented command structures, and prompted calls by Human Rights Watch for accountability for commanders linked to violence in Darfur [232][202].

Humanitarian access and basic services remain constrained by shifting frontlines, aerial attacks, sieges, and opportunistic criminality [202][236]. International pressure rose in early June 2026: Quintet actors called for a humanitarian truce and an inclusive civilian transition after consultations (3–5 June), and the Sudanese executive's 7–9 June 2026 communiqué called for acceptance of a Quartet humanitarian truce, unconditional humanitarian access, releases of detainees, and removal of armed formations from cities [239][242][212]. US senators introduced legislation on 11 June 2026 to sanction individuals and entities accused of supplying weapons or obstructing efforts to end the war and humanitarian access [242]. These dynamics—escalating drone/aerial strikes, defections, intercommunal clashes, and continued arms and resource flows—are driving population displacement, elevating civilian protection risks, and constraining humanitarian operations across Sudan [236][202][232][242].

Security forces and law enforcement

RSF and SAF continue to contest control across multiple states: the RSF has held large parts of West Darfur since mid-2023 and seized key towns and bases in Kordofan, including El Fula and Babanusa [252][221][226]. External state sponsorship has substantially bolstered belligerent operational capacity, with the UAE backing the RSF and Egypt and Saudi Arabia broadly supporting the SAF, while illicit gold-smuggling and external financing have helped sustain fighting; UN Panel estimates place RSF gold production at roughly 10

tonnes in 2024 [186].

Human Rights Watch and other rights actors report that neither main party has taken credible steps to investigate or prosecute members of their forces for serious abuses, and have warned against amnesties that would entrench impunity while calling for defecting RSF commanders to be held accountable [232][217][211]. The ICC's prosecutorial work remains focused on Darfur, with investigations reported to be accelerating as of January 2026, and regional and extra-judicial accountability avenues are emerging, including a universal-jurisdiction case filed in Kenya and calls from rights groups for expansion of international jurisdiction to address alleged war crimes across Sudan [223][232].

The ICRC has expanded logistical links—including the first Port Sudan–Khartoum flight—to deploy specialized personnel and has urged all parties to facilitate safe, unimpeded humanitarian access and to spare civilians and civilian infrastructure in line with international humanitarian law [216]. Security incidents and the involvement of armed actors have disrupted health services and constrained outbreak and humanitarian responses, complicating access for response teams in conflict-affected areas [257][224]. Assistant Secretary-General Martha Pobe reported on 9 June 2026 that the Sudan war's regional spillover includes repeated cross-border incursions and drone strikes affecting Chad, placing major strain on host-state resources [200].

Risks affecting social stability

FEWS NET reported in May 2026 that Emergency (IPC Phase 4) outcomes are expanding and some populations face Catastrophe (IPC Phase 5), with a credible risk of Famine in parts of North Darfur and South Kordofan—notably Dilling, western Nuba Mountains, Kadugli, Um Baru, Kerno, and At-Tine—through May–September 2026 [202]. Several RSF drone strikes killed civilians in At-Tine and Kerno, further disrupting commerce and compounding food insecurity in these high-risk areas [202][190].

Large refugee flows continue to strain neighbouring states: UNHCR recorded 927,915 Sudanese refugees in eastern Chad as of 17 May 2026, while UNOCA reported an overall influx of over 900,000 Sudanese refugees and 300,000 Chadian returnees into Chad since April 2023, exacerbating pressure on host-community resources, markets, and social cohesion [191][200]. Heavy militarization, rising intra-group clashes, and the fragmentation of command structures are undermining localized stability and aid access, while negative coping strategies and protection risks are amplifying social tensions in humanitarian hotspots [202][221].

Legal & Policy

Legal and policy framework

The international legal framework governing accountability for the Sudan conflict rests on several overlapping obligations and mechanisms. Human Rights Watch stated that Sudanese authorities are obligated under international law to investigate, prosecute and punish atrocity crimes, and that using amnesty to shield perpetrators from accountability is incompatible with those obligations [305][289]. The Independent International Fact-Finding Mission on Sudan concluded in February 2026 that the RSF committed crimes against humanity, war crimes and genocide in and around El Fasher, North Darfur, and highlighted that its findings are essential to accountability efforts [296]. The ICC has been investigating crimes in Darfur since 2002 and, according to ICC Deputy Prosecutor Nazhat Shameem Khan, was intensifying investigations in January 2026 into alleged crimes in Al Geneina and El Fasher, while its mandate remains limited to Darfur by the 2005 Security Council referral [305][297][293]. Human Rights Watch urged Security Council members and the Coalition for Atrocity Prevention and Justice — formed in February 2026 by the UK, Germany, Ireland, the Netherlands and Norway — to push for expansion of ICC jurisdiction beyond Darfur and to oppose amnesties that would grant impunity [305][297][293]. In early June 2026, Legal Action Worldwide and partner organisations filed a complaint in Kenya invoking universal jurisdiction and seeking investigations into alleged RSF war crimes and crimes against humanity committed between April 2023 and March 2025, asking Kenyan prosecutors to consider charges under domestic law [324][322][328]. Human-rights monitors documented escalating drone strikes that killed at least 33 civilians in North Kordofan in the seven days before 8 June 2026 and called for independent investigations and adherence to IHL [335]. The ICRC has called on all parties to facilitate safe and unimpeded humanitarian access and to spare civilians and civilian infrastructure in accordance with international humanitarian law, and has expanded logistical capacity inside Sudan — including a resumed Port Sudan–Khartoum ICRC flight — to support response operations [309][306]. The Sudan Humanitarian Fund's 2026 First Standard Allocation committed USD 40 million to restore integrated life-saving health, WASH and nutrition services in high-severity and hard-to-reach localities, while prioritising national and local partners to support humanitarian continuity and localisation efforts [290][301].

Humanitarian impact of legislation

National and international legal and policy environments have direct consequences for humanitarian operations on the ground. Attacks on key transport infrastructure and intensified drone strikes have sharply constrained humanitarian access in Darfur and Kordofan, including a drone strike that damaged the Ardamata Bridge in El Geneina in mid-June 2026 and reported destruction of bridges between Kadugli and Dilling in South Kordofan, threatening primary aid corridors into the region [332][310][336]. Monitoring bodies continue to document persistent attacks on health care and civilian infrastructure throughout the conflict [309][306]. Humanitarian cash programming faces legal and administrative barriers: practitioners report that authorities' approval processes, financial regulations and other administrative impediments delay or limit Multi-Purpose Cash Assistance, while the fragmentation of the formal banking system, liquidity shortages and collapsed services further complicate CVA delivery and reach [273][321][288]. Between January 2024 and April 2026, DTM recorded 766,910 individuals crossing back into Sudan from neighbouring countries, generating large-scale return dynamics that complicate protection, documentation and service delivery planning [294]. Cross-border monitoring and IOM findings show documentation gaps among returnees — about 12% reported crossing without any form of documentation, with strong variation by crossing point — limiting access to legal rights and assistance for a notable share of returnees [314]. A.U.S.

Senate bill introduced on 11 June 2026 (the PEACE in Sudan Act) would expand sanctions authorities and require assessments for terrorist designations, including sanctioning actors accused of obstructing humanitarian assistance, creating potential legal and compliance risks for aid delivery and funding channels [331].

International agreements impacting the crisis

Several multilateral processes and agreements have shaped the international response to the Sudan crisis. Following Quintet consultations in Addis Ababa (3–5 June 2026), the United States, EU members, the African Union, IGAD, the League of Arab States and the UN issued a joint statement backing a rapid, inclusive civilian-led political process, urging a humanitarian truce leading to a permanent ceasefire, endorsing the Berlin Principles adopted at the 15 April Berlin conference, and calling for a comprehensive civilian dialogue to conclude within roughly six months [276][347]. A final communiqué issued on 7–9 June 2026 urged the SAF and RSF to accept a Quartet-proposed humanitarian truce, allow unconditional humanitarian access, release detainees, remove armed formations from major cities and adopt accountability measures – including mechanisms to address war crimes, crimes against humanity and genocide [274]. The Independent International Fact-Finding Mission's February 2026 findings were scheduled for discussion in an Extraordinary Institutional Debate on 15 June 2026, and stakeholders urged the UN Security Council to translate those findings into concrete protection and accountability actions [296]. The UN system has repeatedly called on parties to protect civilians and facilitate rapid, safe, unhindered and sustained humanitarian access, while UN agencies warned of spill-over effects on neighbouring Chad and severe strain on health services from attacks on infrastructure and key aid routes [278][310].

Limitations of current policies

Existing legal and policy frameworks leave significant gaps that undermine humanitarian access, human rights protection and crisis resolution. Humanitarian operations face pervasive administrative and operational barriers – including access restrictions, checkpoints, movement limitations, logistical bottlenecks, fuel shortages and administrative interference – that repeatedly disrupt supply chains and constrain delivery of in-kind and cash assistance [273]. Markets at state level remain largely functional while purchasing power is the primary constraint for households, and affected populations show a strong preference for cash assistance, underscoring the mismatch between needs and restrictive modality policies [273][321]. Large information and coordination gaps – notably the absence of regularly updated mapping of gathering sites and IDP locations – complicate needs assessments and risk excluding populations in unofficial sites from assistance [282][277][279][284]. Humanitarian coverage is patchy, with weak coordination networks and a near-total absence of sustained response in localities such as West Kordofan, East Darfur and Ghubaysh, aggravating protection, health and WASH crises on the ground [277][279][284]. The health system suffers deep governance, funding and workforce gaps – including collapsed oversight, near-total reliance on external funding and severe staff shortages – which limit recovery, service continuity and the ability to absorb humanitarian inputs [275]. Donor funding cuts are already forcing service reductions: the IRC warned that cuts could close 34 IRC-supported health facilities by July 2026 and reported that water trucking has ceased for 100,000 people in Central Darfur, increasing outbreak risks and service loss [337]. Serious accountability shortfalls persist: human rights monitors report a lack of credible investigations and the use of amnesties or impunity that undermine obligations to prosecute atrocity crimes, while international jurisdictional mandates such as the ICC remain geographically limited to Darfur, leaving accountability gaps across the country [305][289][297]. Published analyses and funding strategies call for removing administrative barriers to cash, expanding MPCA, improving mapping and coordination, restoring health governance and financing, prioritising national and local partners with capacity building, and pursuing expanded accountability mechanisms to address impunity [273][275][301][337][305].

Demography

Demographic overview

As of 31 May 2026, Sudan hosted 868,075 refugees, of whom Ethiopian refugees numbered 67,062 (approximately 8% of the total) [377][356]. Key registered statistics for Ethiopian refugees (UNHCR, 31 May 2026):

- Total individuals: 67,062; total households: 20,322; registration coverage ~92%; 4,610 households registered at household level (unregistered ~7%).
- Age/sex breakdown (registered): adult males 33%, adult females 26%, male children 18%, female children 17%; persons with disabilities 1,009 (~2%).
- Camp vs. out-of-camp: 72% in camps, 28% out of camps [356][377].

Between January 2024 and April 2026, IOM DTM recorded 766,910 individuals crossing back into Sudan from neighbouring countries, with the majority returning from Egypt (82%) and smaller proportions from Chad (18%), Ethiopia and Eritrea (<1% each) [363]. From August 2025 to April 2026, 135,179 individuals were recorded returning from Chad, with 66% using Um Dukhun and 34% using Adekonk-Ag Geneina as crossing points [360]. Among those in the Chad corridor, 58% were Sudanese and 42% non-Sudanese (of whom approximately 77% Chadian and 18% Central African); children under 18 comprised approximately 56% of travellers while those aged 18–59 comprised approximately 44%, with a higher share of females among adults (32% female vs. 12% male in the 18–59 group) [360]. DTM found that 64% of individuals crossing into Sudan intended to return to their places of origin within three months, 24% planned to move elsewhere within Sudan, and 12% planned onward movement outside Sudan [363].

Household structure among recent returns and refugee populations is marked by high shares of female-headed households: 91% of households among returnees at Chad border entry points in May 2026 were female-headed (IOM), and UNHCR data indicate approximately 38% women-headed households among registered Ethiopian refugees in Sudan [367][356].

Internal displacement sites show a demographic profile dominated by women and children with marked male absence: Al Nahil IDP camp (established 5 March 2025) hosted an estimated 18,020 people (3,260 households) and was reported to be 85–90% women, with the remainder mainly children and few men [359]. In West Kordofan (Ghubaysh) gathering sites, assessments reported a high proportion of women and children among IDPs; local authorities provided an unverified estimate of approximately 68,000 IDPs in the area, while six gathering sites officially hosted more than 2,000 households (~10,000 people) at the time of assessment [370].

Sudan's population is characterised by broad ethnic, cultural and religious pluralism — including Arab, Nubian, Darfurian, Nuba, and multiple Nilotic and other African groups — contributing to significant subnational demographic variation relevant for humanitarian response planning [381].

Third country nationals

According to IOM, 1,329,732 people crossed into Chad since the onset of the Sudan crisis, including 400,914 Chadian returnees as of 31 May 2026 (projected to reach 405,000 by June 2026); these returnees lack resources and have urgent needs for food, WASH, shelter, NFIs, healthcare and protection [367][354]. By April 2026, Chad was hosting approximately 1.2 million Sudanese refugees, including 927,000 arrivals since April 2023, creating sustained pressure on host resources and services [375]. IOM biometric registration data in Chad captures returnee demographics of 58% female, 69% children, 4,809 pregnant women and 14,012 lactating women, highlighting heightened needs for maternal and child health, protection and WASH services [367].

UNHCR reported nearly 989,200 Sudanese in refugee-like situations in 2025, with major host countries being Libya (388,100), Egypt (264,700), Chad (181,700), South Sudan (76,000) and Uganda (27,600) [365]. Sudanese in Libya face arrests, hate speech and heightened fear, with estimates of hundreds of thousands — some reports citing more than half a million — prompting calls for legal and humanitarian protection and voluntary repatriation support [358].

Socio Cultural

Socio cultural overview

Sudan is characterized by broad ethnic, cultural and religious pluralism—including Arabs, Nubians, Darfurians and diverse Nuba/Blue Nile groups—and long-standing marginalization of peripheral regions (Darfur, South Kordofan, Blue Nile, Eastern Sudan) that has helped transform cultural differences into political divides [424][407]. The 2023–26 war has produced a de facto division of power, with rival authorities imposing influence over different regions and hardening a territorial split by early 2026 [407][442]. The Islamist movement remains a significant political and social force, with networks inside state institutions and broad alignment with the Sudanese Armed Forces, making its inclusion in future political settlements highly contested [410]. On 10 June 2026, the Civil Democratic Alliance (Sumud) called for a humanitarian truce and confidence-building measures, recommending the exclusion of the dissolved National Congress Party and the Islamist movement from future political arrangements [446]. Civil society actors have urged community leaders, religious figures and media to confront hate speech and strengthen social cohesion [446].

Local social structures and a tradition of mutual aid have become primary frontline providers of humanitarian assistance, with mutual aid groups and platforms such as the Collective Sudan coordinating cash and voucher assistance across affected areas [416][428]. Religious community leaders (sheikhs) play influential roles in representing households, resolving disputes and coordinating with authorities and humanitarian actors [428]. Women-led organizations continue to lead humanitarian response, protection and resilience efforts while calling for meaningful participation of women and girls in decision-making processes [406]. Community-based health care, local initiatives and professional networks have partially sustained basic health services amid institutional breakdown, relying on social capital but remaining limited in resources and sustainability [411]. Assessment teams in West Kordofan in early June 2026 engaged community leaders and women's representatives to capture population dynamics, service gaps and protection concerns in IDP-hosting localities such as Ghubaysh [434].

Gender dynamics

On 9 June 2026, the UN Special Representative on Sexual Violence in Conflict highlighted that women in Sudan have been subjected to public rape as a tactic of terror and remain largely invisible in high-level negotiations despite leading local peace initiatives [426]. The Regional Platform for Support to Women-led Organizations, established in 2025 with UNHCR and UN-Women, calls for women's full, equal and meaningful representation in humanitarian coordination, peace negotiations and political processes, and for direct, sustainable financing to strengthen women-led responses [406]. After four years of conflict, women and girls continue to face rising risks of forced displacement, conflict-related sexual violence, deteriorating livelihoods and limited access to basic services [406].

In the Abyei area, UNISFA reported women's direct participation in a biannual migration conference rose from 19% in 2023 to 41% in 2025, yet women remain underrepresented in local security mechanisms (7–16%) and are almost absent from traditional courts [404]. Assessments in West Kordofan document harassment, looting and incidents of physical and sexual abuse during transit to safer areas, indicating heightened protection risks for women and girls on the move [434]. Cross-border monitoring found pregnant or lactating women constituted 42% of returnees from Chad, reflecting high care and service needs among mobile populations [431]. Women-headed households face specific economic barriers in covering food and basic costs, increasing their vulnerability and shaping prioritization of assistance modalities [423]. Cash and voucher assistance is widely preferred—over 90% of affected households identify cash as the most dignified form of support—with local responders including mutual aid groups and women-led organizations serving as primary frontline providers [428].

Health sector monitoring shows Sudan emerged in 2025 as a context with recorded arrests of health workers, undermining service delivery; gender data are incomplete but men are more frequently recorded among detainees where reported [448]. UNHCR reporting (31 May 2026) shows refugee populations hosted in Sudan include substantial female shares (Ethiopian refugees approximately 45% female), underscoring the need for gender-sensitive protection and services in refugee and host communities [433].

Impact of socio cultural drivers on aid

Aid delivery in Sudan is significantly shaped by social structures and cultural dynamics. Recipients reported limited and mostly indirect engagement with aid providers, often mediated through community committees or sheikhs, and many said they were not consulted on programme design, modalities or distribution arrangements [428][416]. Authorities imposed regulatory restrictions on delivery mechanisms—often permitting only digital transfers—which constrained choice of modality and affected accessibility for some groups [428]. Centralised registration and distribution points were described as stressful and inaccessible for people with disabilities, with recipients recommending door-to-door approaches and tailored outreach for vulnerable groups [428]. Field assessments that used consultations with women representatives and gender-disaggregated focus groups demonstrated that meaningful community engagement increases trust and programme relevance [439].

A West Kordofan assessment (03/06/2026) documented SGBV incidents during displacement, a lack of safe and dignified facilities for women and girls, and recommended safe spaces, confidential case management, psychosocial support and distribution of menstrual kits [434]. The same assessment found women primarily engaged in small, informal daily work (tea selling, laundry), and noted that IDPs in Ghubaysh are closely integrated with host communities, implying assistance should prioritise shared services and avoid interventions that could fuel host-displaced tensions [434].

Community and informal support networks—including community health care initiatives and mutual aid groups—have been central to maintaining basic services and local responses, reducing but not replacing the need for formal humanitarian support [411][428]. Regional civil society statements continue to call for women's full, equal and meaningful representation in humanitarian coordination, direct financing for women-led organisations, and strengthened GBV services and accountability, reflecting local advocacy priorities that directly affect the acceptance and design of aid interventions [406].

Environment

Environmental conditions

Sudan is experiencing a long-term decline in rainfall alongside rising temperatures and increasingly irregular rainy seasons, heightening the country's combined drought and flood risk [469]. Climate risks are concentrated in the central agricultural and pastoral belt between roughly 10°–16°N, which is also the heart of rain-fed agriculture and grazing livelihoods, with historical major floods recorded in 1946, 1988 and 2019 [469]. Forecasts in mid-2026 signal a high likelihood of El Niño developing into 2026–27, raising the probability of below-normal Sahel rainfall during June–September 2026 with adverse implications for soil moisture and cropping seasons [498][533]. Extreme heat and associated water stress are already worsening food security and production, with assessments indicating nearly 25 million people in Sudan are food-insecure [514]. Conflict and institutional weakening — including the erosion of capacity at the Ministry of Irrigation and Water Resources, staff migration and loss of expertise — have degraded water management and elevated water as a national security concern, as noted on 11 June 2026 [516]. Post-conflict damage and neglect of irrigation and drainage infrastructure (including systems linked to the Sennar reservoir), together with looted agricultural machinery, further reduce irrigation effectiveness, increase salinity risks and constrain agricultural recovery [506][469]. Gaps in climate monitoring and the need to modernize observing networks and build integrated early-warning systems have been identified as priorities to manage drought and flood hazards more effectively [469].

Environmental challenges

Access to safe water is severely compromised in multiple locations. A West Kordofan (Ghubaysh) assessment in early June 2026 reported limited supplies, 2–3 hour queues, water contamination confirmed by testing, and some households walking up to four hours to fetch water [484]. An assessment of Al Nahil IDP camp (site visit 6 May 2026) found a single manual handpump serving about 18,020 people that yields poor-quality, high-salinity water; previous water trucking of approximately 150 barrels/day (~30 m³) covered only one jerrycan per household before operations stopped, leaving residents reliant on purchased water at reported prices of 4,000 SDG per barrel or 200 SDG per jerrycan [483]. Municipal waste services have halted in some areas since the start of the conflict, leading to open dumping on city outskirts and increased vector and environmental pollution risks, as observed in Ghubaysh on 23 May 2026 [484]. Waterborne disease risk is elevated and prioritized in humanitarian planning: the Sudan Humanitarian Fund 2026 WASH strategy allocates \$13,000,000 for cholera/AWD/Hepatitis E preparedness and response, including water quality monitoring, chlorination across the water chain, WASH kit distribution and disinfection activities [512]. Accelerating desertification and degraded pastures further compound pressure on water resources across the country [469].

Agricultural calendar

Sudan's main unimodal rainy season runs June–September and underpins rainfed cereal and pastoral production across the country [464]. Seasonal forecasts issued in June 2026 indicate a likely delayed onset and below-average rainfall across parts of Sudan associated with a developing El Niño, raising the risk of moisture stress during critical crop growth stages, while short-term outlooks for 11–17 June 2026 forecast suppressed rain and abnormally hot temperatures (maximums exceeding 35°C) that may exacerbate crop and pasture stress [464][465][500][474]. Humanitarian analyses note the May–September lean season has begun and that Emergency outcomes are expanding, with a credible risk of Famine in parts of North Darfur and South Kordofan if conflict, market disruption and access constraints cut off food and assistance [477]. On 8 June 2026, Finance Minister Dr. Gebreil Ibrahim chaired a Port Sudan meeting with AfDB, WFP and other partners to secure urgent financing for the 2026–2027 summer agricultural season — prioritizing improved seeds, fertilisers, fuel and pesticides — and to review AfDB-supported wheat projects and the Port Sudan Water Project implemented by UNICEF [479]. The African Development Bank and WFP launched the four-year BOOST project (22 May 2026) to strengthen rain-fed production in Sennar and Blue Nile, aiming to support more than 230,000 farming households and produce nearly one million metric tons of cereals and pulses over four years [497][491].

Large irrigation initiatives described as the "island" project have released water from the Sennar reservoir to start the season and aim to expand irrigated area (claims up to 1.2 million acres), while observers caution that success depends on urgent canal and drainage maintenance and substantial, timely financing to farmers [506].

Environmental risks and mitigation

Seasonal outlooks from WMO/GHACOF indicate a high probability of below-average June–September (JJAS) rains across many areas of Sudan and above-normal temperatures in northern Sudan, increasing the risk of moisture stress, crop heat stress and reduced fodder availability during JJAS 2026 [533]. Short-term weather data for 11–17 June 2026 showed rainfall surpluses over the previous 30 days in parts of Sudan, while abnormally hot maximum temperatures exceeding 35°C were forecast for the outlook period alongside lighter near-term rain [474][470]. A June 2026 Sudan climate memorandum recommends prioritizing climate observing networks, early-warning monitoring (including orbital separation/monsoon timing) and use of seasonal forecasts to guide agricultural and pastoral investments and planning [469]. UN guidance on post-conflict rubble management urges people-centred circular approaches during reconstruction — prioritizing damage and hazard assessments, segregation and safe handling of hazardous materials (e.g., asbestos, UXO), occupational safety, and maximizing reuse/recycling to reduce environmental harm and emissions from disposal and transport [532]. Taken together, practical mitigation priorities for Sudan in mid-2026 include strengthening climate observing and early-warning systems, applying seasonal forecasts to agricultural and pastoral planning, and adopting sustainable rubble management and hazardous-material controls during reconstruction to limit environmental damage and health risks [469][533][532].

Conflict

Overview of the ongoing armed conflict

The current war began on 15 April 2023 between the Sudanese Armed Forces (SAF) and the Rapid Support Forces (RSF) and has hardened into a de facto territorial partition that shifted from the capital region to Darfur and then to Kordofan by early 2026 [563]. Fighting is high-intensity and increasingly characterised by drone strikes, air raids, rocket fire, shelling, bombardments and heavy artillery in populated areas, with allied armed groups further fragmenting the conflict [628][660][652]. Drone and airstrike warfare has intensified civilian harm and compounded access restrictions for humanitarian agencies attempting to deliver aid [652][660]. Reports also document widespread sexual and gender-based violence used as a tactic in the conflict [628].

Key recent facts and figures:

- More than 12 million people displaced and credible estimates of up to 400,000 dead since the conflict's outbreak [563].
- Independent conflict analyses and UCDP/PRIO assessments recorded a dramatic rise in one-sided violence in 2025, with sieges and massacres in El Fasher estimated at around 60,000 deaths and driving much of the 2025 surge in civilian fatalities [569][586].
- UN and local monitors reported at least 880 civilians killed by drone strikes between January and April 2026 [664][589].
- A Darfur victims' rights group documented 470 deaths and 187 injured from air raids and drone attacks between March 2024 and January 2026, and reported use of foreign-made drones (including Iranian Mohajer/Ababil models and Turkish Bayraktar systems) in strikes that have destroyed infrastructure and disrupted farming and trade [660].
- Between 8–15 June 2026, local reporting documented renewed strikes and civilian casualties: drone strikes in El Obeid (North Kordofan) on 12 June killed at least 19 civilians [651], and a shooting at a social event in South Darfur on 13 June killed at least three people [650].

Conflict stakeholders

The conflict is principally between the SAF and the paramilitary RSF, with allied armed movements and regional militias contesting territory across Khartoum, Darfur, Kordofan and other states [560][581]. In Darfur and other fronts, multiple non-state groups are active, including the Sudan Liberation Movement, the Justice and Equality Movement (JEM), and components of the Sudan Revolutionary Front (JEM, SLA-AW, SLA-MM, SPLM-N), as well as janjaweed and various community militias [560][592].

External state sponsorship is central to the conflict's dynamics: the UAE is identified as a principal backer of the RSF, while Egypt and Saudi Arabia broadly support the SAF, with Türkiye, Qatar and Russia moving between sides [563][578][574]. A UN Panel of Experts estimated the RSF produced roughly 10 tonnes of gold in 2024, creating lucrative self-financing incentives and cross-border illicit trade that sustain its operations [563]. Human Rights Watch and other monitors documented the recruitment and deployment of hundreds of Colombian private military contractors allegedly hired via an Abu Dhabi-based security company to fight alongside the RSF [574][575]. Conflict monitors (ACLED) have also reported defections and intra-coalition tensions within RSF ranks, indicating growing fragmentation and violent competition over remaining war spoils [574][575].

On the accountability and legislative front, US senators introduced bipartisan legislation on 11 June 2026 to expand sanctions against individuals and entities supplying weapons, recruiting fighters, smuggling resources or obstructing humanitarian assistance in Sudan [661]. Human Rights Watch and other accountability advocates caution that defections and political arrangements risk impunity for alleged perpetrators and have urged investigations and prosecutions of commanders accused of abuses [619][641]. International partners issued a joint statement in Addis Ababa on 9 June 2026 backing a civilian-led, time-bound national dialogue and roadmap toward a civilian transition [607][581][626].

Impact of the conflict on the population

By early June 2026, large-scale displacement and acute humanitarian needs persisted across Sudan. Key displacement figures include:

- 33.3 million people required humanitarian assistance [561].
- 8.9 million people remained internally displaced in May 2026 (a 23% decline since January 2025), while return movements rose to 4.2 million, with most returns reported to Khartoum (nearly 2 million) and Al-Jazirah (1.2 million) [546][545].
- Cumulative crossings from Sudan to neighbouring countries since April 2023 exceeded 3,018,640, with Chad and Egypt among the largest recipients [549][550]. IOM and UNHCR data show 1,329,732 individuals crossed into Chad since the crisis began, of whom 400,914 were Chadian returnees as of 31 May 2026, with IOM projecting Chadian returnees to reach about 405,000 by June 2026 [554].

Displacement sites and host communities face severe multisectoral gaps — food, cash, shelter, WASH, primary health care and protection — with concentrated needs in assessed sites such as Khazan Jedeed, where three camps host roughly 45,500 people living in precarious shelter with extremely limited safe water, non-functional sanitation, medicine shortages and almost no sustained humanitarian presence [634].

Acute food insecurity is expanding: FEWS NET projects 40–45% of the population will need food assistance by December 2026, and national analyses warn Emergency (IPC Phase 4) outcomes are widening with some areas at risk of Catastrophe (IPC Phase 5), notably in North Darfur and parts of Greater Kordofan and South Kordofan [544][546]. Health and WASH risks are elevated — cholera outbreaks have been reported in displacement sites and suspected large-scale cholera was noted in parts of West Kordofan — while fragile markets, rising food and fuel prices, and degraded transport and storage constrain both commercial and humanitarian access [634][546].

Ongoing hostilities, including drone strikes and attacks on markets and key routes, have caused civilian casualties, damaged infrastructure and impeded humanitarian access, prompting international calls for a humanitarian truce to enable safe, sustained assistance [557][567][553][626].

Peace negotiations

Since fighting broke out in April 2023, international diplomacy in June 2026 has focused on securing a humanitarian truce as a stepping stone to a civilian-led political transition [626][607][581]. Announced in Nairobi in May 2026, a Nairobi roadmap proposes a three-track process combining humanitarian measures, a renewable internationally monitored ceasefire, and a political transition focused on state reform, championed by a civilian anti-war bloc that includes the Somoud coalition and the Sudan Liberation Movement [564][593]. The Nairobi text seeks to exclude SAF and RSF from the political track while involving them in ceasefire talks, and proposes barring leaders of the Islamist movement (including the former National Congress Party) from political participation — a contentious approach that analysts warn could complicate prospects for a durable settlement [564].

The Quintet (UN, African Union, European Union, League of Arab States, IGAD), backed by Western states, held consultations in Addis Ababa in early June 2026 and issued joint statements supporting a time-bound, inclusive civilian-led dialogue intended to produce a roadmap toward civilian rule within roughly six months [607][580][626]. Regional and international mediation tracks active alongside Nairobi include the Quad (United States, Saudi Arabia, UAE, Egypt), the Quint mechanism, and outcomes from the April 2026 Berlin conference, while states such as the UAE have publicly urged a humanitarian truce and the launch of a civilian political track [564][596][580]. Sudanese civilian actors, including the Civil Democratic Alliance (Sumud), publicly called on the SAF and RSF to accept a proposed humanitarian truce, release detainees, implement prisoner exchanges, withdraw forces from major cities, and reopen civic space as confidence-building measures ahead of negotiations (Nairobi meetings, 7–9 June 2026) [593].

Major obstacles highlighted across initiatives include the unresolved debate over Islamist inclusion, the enduring influence of former regime networks in state and security institutions, the risk that a ceasefire without a broader political settlement will institutionalize military fragmentation, and the potential need for international measures against actors who seek to undermine a civilian transition [564][607][580].

Conflict outlook

The Sudanese conflict remains fragmented, with substantial RSF territorial control reported in country-level assessments, and the war has entered its fourth year with continued high-intensity dynamics as of early June 2026 [589][677]. Local fighting has turned areas such as Khazan Jedeed (East Darfur) into major displacement hubs, and tens of thousands fled following the capture of El Fasher in October 2025 [634]. Samud's Executive Office (meeting 7–9 June 2026) described ongoing escalation and called for an end to the April 15, 2023 hostilities [677].

Sudan's crisis is embedded in a tightly interlinked Horn of Africa conflict system — characterised by refugee flows, gold-smuggling routes and external sponsorship — which raises the risk of further spillover and makes unilateral de-escalation less likely without regional diplomatic action and tighter arms-embargo enforcement [563][589]. Humanitarian impacts are acute: national data indicate roughly two in five people face crisis-or-worse food insecurity, with nearly 200,000 in catastrophic hunger, and forecasts warn food insecurity will worsen from mid-2026 as El Niño affects rainfall patterns, while humanitarian supply chains are already fracturing under regional shocks [646][638]. Overall, the outlook is high risk for further intensification and regional entanglement; a negotiated settlement appears unlikely in the near term absent coordinated regional diplomacy, enforcement of embargoes, and measures to restore humanitarian access [563][589][677].

Displacement

Displacement overview

As of May 2026, an estimated 8.9 million people remained internally displaced inside Sudan, a 23% decline from January 2025, reflecting large ongoing returns throughout 2025 with around 3.6 million people reported returning to Sudan that year [732][736][728][749]. DTM recorded 766,910 individuals crossing back into Sudan from neighbouring countries between January 2024 and April 2026, with 82% returning from Egypt and 18% from Chad (Ethiopia and Eritrea each under 1%) [683].

Recent displacement dynamics remain concentrated in frontline areas: nearly 60,000 people were displaced in Blue Nile since January 2026, with multiple localized displacements in late May 2026 in Kordofan and South Darfur [732]. Movements from Chad largely reflect protracted mobility rather than new flight – 72% of arrivals reported no recent displacement and only 26% reported displacement after April 2023 – with origins concentrated in Central Darfur (41%), West Darfur (34%), and South Darfur (17%); Khartoum is the single largest state of origin for cross-border returnees overall (32%) [681].

Movement intentions remain highly uncertain: among those intending to return, 70% did not know how long they would stay, and primary reported drivers for crossing back included improved security (approximately 50%), livelihood or business opportunities (24%), family visits (24%), and legal difficulties abroad (13%) [681][683]. Many returns recorded in 2025 occurred under pressure and into precarious conditions, while global resettlement and complementary pathways contracted, reducing available durable solutions for refugees [728][749][712]. Cross-border and internal return movements have increased pressure on host areas and services – for example, arrivals into White Nile State from West Kordofan since October 2025 are straining limited capacities [704].

Living conditions of the displaced population

Assessments in Khazan Jedeed (Sha'ria locality, East Darfur) in early May 2026 found severe humanitarian conditions among newly arriving displaced populations, critical gaps in basic services, and a limited humanitarian presence amid continued population influx [685][693]. The three assessed IDP camps – Al Nahil, Al Salam, and Al Wahida – together host approximately 8,000 households (around 45,500 people), with most displaced households having arrived after April 2025; broader estimates indicate more than 80,000 IDPs concentrated in the area and several hundred thousand displaced across the wider Khazan Jedeed administrative unit [693]. Shelter conditions are extremely precarious, with most shelters makeshift (branches, straw, fabric) and insufficient to provide protection from climatic conditions or basic privacy [693].

Major multisectoral needs were identified across WASH, health, shelter, food security, and nutrition. Cholera outbreaks inside displacement sites in 2025 caused confirmed deaths linked to lack of medicines, IV fluids, and sanitation infrastructure [693]. Key WASH indicators from community-level assessments show 56.93% of latrines reported as non-functional, 15.33% of facilities with cleanliness problems, 10.22% with safety concerns, 8.03% with privacy concerns, and 3.65% with accessibility issues [753]. Protection emerged as the largest need among the displaced (16.38%), compared with 8.4% among returnees and 8.6% among host communities; documented protection concerns include family separation, unaccompanied and separated children, child-headed households, sexual violence against girls, and lack of assistive devices for people with disabilities [693][753]. Non-food items linked to dignity – mattresses, hygiene and MHM kits, diapers, cooking and food-storage items – were repeatedly cited as priority needs [753].

In West Kordofan (Ghubaysh), a 3 June 2026 assessment reported lack of updated IDP mapping, high local NFI prices limiting access, non-existent or inadequate shelter exposing IDPs to rain and heat, reliance on private clinics for healthcare, widespread child malnutrition, and limited, largely informal livelihood opportunities [680]. Across border areas, humanitarian actors including CNARR, UNHCR, IOM, and partners provided WASH, NFIs, shelter support, and protection services in May–June 2026, with UNHCR and CNARR assisting an estimated 67% of refugees to move from spontaneous sites into established camps in Chad as of 9 June 2026 [688][686]. Overall, assessments highlight rapidly growing needs across protection, WASH, shelter, health, and food and nutrition while humanitarian presence and response capacity remain severely limited relative to demand [685][693][680].

Population movements

Between January 2024 and April 2026, DTM recorded 766,910 individuals crossing back into Sudan from neighbouring countries [683]. Of those, 64% intended to return to their place of origin within three months, 24% planned to move elsewhere within Sudan, and 12% planned onward movement outside Sudan – with intended destinations including Chad (49%), Central African Republic (38%), and Egypt (4%) [683]. Individuals crossing from Ethiopia into Sudan mainly originated from Gedaref (36%), Khartoum (24%), and Kassala (20%), with only 24% reporting displacement after April 2023 [681].

Internal returns within Sudan increased to 4.2 million (a 6% month-on-month rise), with most returns to calmer areas such as Khartoum (nearly 2 million) and Al-Jazirah (1.2 million); 83% of these returns were from locations of displacement within Sudan [732][736].

By April–May 2026, Chad hosted approximately 1.2 million Sudanese refugees, with roughly 927,000 arrivals since April 2023. A total of 23,800 arrivals were recorded in the first four months of 2026, compared with 55,600 in the same period in 2025, indicating a significant slowdown in new arrivals [690][689]. IOM and UNHCR data show cumulative crossings into Chad since the crisis began at approximately 1,329,732, including roughly 400,914 Chadian returnees as of 31 May 2026 [695][700]. IOM monitoring at the Adré and Tiné entry points recorded 845 arrivals in May 2026 (780 at Adré, 65 at Tiné), a 39% decrease compared with April 2026 counts at those two points [686]. UNHCR reports 67,062 Ethiopian refugees in Sudan as of 31 May 2026, representing approximately 8% of Sudan's total refugee population of 868,075 [711][727].

Drivers of displacement

Armed conflict and targeted violence have been the principal driver of displacement. The Rapid Support Forces (RSF) were found by the Independent International Fact-Finding Mission to have committed crimes against humanity, war crimes, and genocide against non-Arab communities during the October 2025 siege and takeover of El Fasher, fueling mass displacement and severe protection threats [761]. By the end of 2025, Sudan remained the world's largest internal displacement crisis, with approximately 9.1 million internally displaced persons and large cross-border refugee outflows of around 952,700, even as some 3.6 million returns were recorded — many occurring under pressure and into fragile conditions [735][728][716]. Neighbouring Chad had received nearly 920,000 refugees from Sudan since April 2023, reflecting sustained cross-border movements and significant host-country strain [690].

Environmental hazards compounded conflict-driven displacement: flooding and heavy rains between April and November 2025 displaced nearly 30,200 people and repeatedly re-displaced conflict-affected populations, while drought and climatic stress have exacerbated resource scarcity and community tensions [704][698]. Severe food insecurity and economic collapse further increased displacement risk, with two in five people in Sudan facing crisis or worse levels of food insecurity and nearly 200,000 people in catastrophic hunger, deepening drivers of flight and reducing absorptive capacity for returnees [738]. Returns often occurred to areas with damaged infrastructure, limited basic services, and continued insecurity, raising risks of secondary displacement and failed reintegration [708][716].

Integration

Assessments in Ghubaysh (West Kordofan, 3 June 2026) and Khazan Jedeed (Sha'ria, 5 May 2026) report that IDPs are closely integrated into urban host communities — often hosted within households and relying on markets and informal work — while formal employment opportunities remain limited, with women engaged in small daily activities and many men inactive [680][693]. Schools reopened in Ghubaysh in May 2026, but many displaced children have dropped out because families cannot afford annual school-related costs estimated at approximately 7,000 SDG per year [680]. Access to healthcare is largely market-based through private clinics; nutrition services are absent (no malnutrition treatment centre and no measles vaccination campaign reported in Ghubaysh); and cholera outbreaks in Khazan Jedeed in 2025 caused confirmed deaths, highlighting weak public health provision for both displaced and host populations [680][693].

Water and sanitation services are inadequate: households depend on purchased water and donkey carts, face 2–3 hour queues, water quality testing indicated contamination, sanitation coverage is low, and municipal waste management has ceased, increasing pressure on shared services [680]. Food insecurity is acute — Ghubaysh communities report a catastrophic food situation with no WFP distributions and limited communal kitchen coverage — while displaced households often prioritise children's meals and reduce dietary diversity [680].

Despite these service gaps, assessments note positive social cohesion in urban sites where family links and proximity to markets contribute to perceptions of safety and a preference to remain in situ rather than move to formal camps, suggesting interventions should prioritise shared infrastructure and services benefiting both displaced and host communities [680][693]. Regionally, UNHCR and partners are working to expand refugee inclusion in national systems: a World Bank-funded project is extending social protection coverage to approximately 50,000 Sudanese refugees in eastern Central African Republic, and World Bank-supported biometric ID pilots in Chad have issued 25,000 identity cards to facilitate inclusion in unified social registries [689][724]. IOM is coordinating multisector assistance in Chad — covering WASH, food, protection, health, shelter, NFIs, and education — alongside 928 social-cohesion caravans and coexistence trainings, indicating significant operational support but continued pressure on host systems [700][686]. Key integration gaps include limited formal employment and vocational pathways, unaffordable education costs, weak and overloaded health and nutrition services, inadequate WASH and waste management, and the need for stronger registries, documentation, and expanded safety nets [680][689][724][686].

Humanitarian Access

Humanitarian access overview

Humanitarian access in Sudan remains severely constrained by insecurity, attacks on civilian infrastructure, administrative impediments, and repeated supply-chain disruptions that limit movements across Darfur and Kordofan, increasing logistical costs and delays [795][796]. In April 2026, WFP reached 3.1 million people with food, cash, and nutrition assistance—including 1.4 million in Greater Darfur—but drone strikes, insecurity along key routes, funding shortfalls, and supply disruptions are severely constraining operations and threaten the planned target of 4.4 million people between June and September 2026 [795]. A drone strike on the Ardamata Bridge in El Geneina in mid-June 2026 caused extensive damage, forced suspension of traffic on a critical Darfur corridor, and prompted a U.S. State Department warning that the attack jeopardises deliveries of emergency food and medical supplies to Zalingei and other areas of West and Central Darfur [821].

Funding and operational pressures are forcing service reductions, with reported funding cuts requiring the closure of 34 IRC-supported health facilities in Sudan by July 2026, risking loss of healthcare access for nearly half a million people [785][799]. Health service delivery is sustained largely through community care, local initiatives, and humanitarian actors rather than a functioning national system, with urgent priorities to ensure continuity of primary health care, rehabilitate supply chains for essential medicines, and protect and support remaining health personnel [799].

Cash and voucher assistance is widely preferred and effective where markets function—over 90% of affected households report preferring cash—and is presented as a key modality to bypass commodity supply constraints, though administrative requirements and financial regulations continue to limit cash scale-up in some areas [804][796]. OCHA's Sudan humanitarian financing architecture is responding with pooled funding priorities in 2026, including the Sudan Humanitarian Fund's indicative US\$40 million envelope to support integrated, life-saving services and protection-centred responses for IDPs, host communities, and returnees in the

highest-severity locations [798].

Barriers to humanitarian access

Fighting since April 2023 has produced a massive crisis in Sudan: in 2026, an estimated 33.3 million people require humanitarian assistance and more than 11 million are internally displaced [804]. Access constraints are most acute across Darfur and Kordofan and in high-severity locations prioritised by the Sudan Humanitarian Fund, affecting IDPs, host communities, and returnees across 13+ states [796][798].

Humanitarian access is severely constrained by active insecurity, movement restrictions and checkpoints, damaged infrastructure, fuel shortages, and repeated logistical bottlenecks that impede the transport and delivery of goods and personnel [796]. Regulatory and administrative obstacles compound these challenges: authorities frequently restrict approved delivery modalities (often permitting only digital transfers) and impose approval processes, customs rules, and financial regulations that delay or limit both cash and commodity programming [789][796]. Assessment reports from Khazan Jedeed, East Darfur (May 2026) and Ghubaysh, West Kordofan (June 2026) documented critically limited humanitarian presence, severe service gaps, and the absence of updated population mapping—raising the risk that non-official sites and recently arrived IDPs will be excluded from assistance [800][786].

Protection and social barriers further compound access. Recent assessments recorded widespread WASH deficits (open defecation, unaffordable soap), lack of safe and gender-segregated facilities, and elevated GBV risks that restrict women's and girls' safe access to services and dignity [786]. IOM cross-border monitoring also identifies movement-related protection risks at transit points—including loss or damage of belongings, general insecurity, language barriers, bribery and extortion, and long processing times—that impede safe movement and access to services [812].

Humanitarian access gaps

Access gaps are most acute in East Darfur and parts of West Kordofan. Assessed camp populations in Khazan Jedeed include:

- Al Salam: 9,555 individuals (1,911 HH)
- Al Nahil: 18,020 individuals (3,260 HH)
- Al Wahida: 18,000 individuals (3,000 HH)
- Total assessed population: 45,575 (8,171 HH) [788]

The displaced population profile is heavily weighted toward women and children while many men are absent (killed or away), increasing protection and livelihood vulnerabilities for female-headed households [788]. Critical sector shortfalls across assessed sites include water supplies critically below emergency thresholds with almost no functioning water source inside camps, sanitation at roughly one latrine per 87 households, food consumption reduced to about one meal per day, and food distributions that are irregular—reaching less than half of camp populations and systematically excluding recent arrivals [788]. Health access is severely limited by medicine shortages and active outbreaks of cholera and measles within camps [788]. A real danger persists that populations in non-official gathering sites are excluded from aid due to movement restrictions and inadequate mapping [788][786].

At the system level, Sudan's health sector shows major information, funding, and workforce gaps—collapsed health information systems, near-total dependence on external humanitarian funding, and severe staff shortages producing regional imbalances and reduced service quality [799]. Security constraints and attacks against health personnel further restrict access: arrests and detentions of health workers were reported in 2025, and serious alleged atrocities by armed actors in Darfur—including a large-scale October 2025 assault in el-Fasher—produced mass casualties and impeded humanitarian operations [814][834].

Some alternative delivery measures have been implemented—humanitarian actors began water trucking and handpump rehabilitation in Khazan Jedeed with operations starting around May 2026—but these remain insufficient to close gaps and often operate as parallel, donor-dependent service channels rather than integrated system recovery measures [788][799]. Assessments recommend regular, comprehensive mapping of all camps and gathering sites and sustained advocacy to ensure NGOs can access and assist populations regardless of a site's official status [786][788].

Information & Communication

Information landscape

Humanitarian information systems in Sudan show significant gaps in population mapping, needs data, and coordinated reporting. West Kordofan assessments in early June 2026 note weak or non-existent coordination and the absence of updated, accurate mapping and population figures for gathering sites, complicating humanitarian planning and risking exclusion of people in unofficial sites [845][846]. Reporting reliability is further constrained by access, political restrictions, and funding shortfalls; institutional monitoring is vulnerable to shifts in authority and reduced field presence, while open-source reporting has expanded but diverges from institutional datasets, complicating validation and comparability [858]. Funding cuts and operational constraints are already undermining response and data coverage — the IRC reported planned closure of 34 health facilities in Sudan by July 2026 and that drone strikes accounted for over 80% of civilian deaths between January–April 2026, both affecting monitoring capacity and service continuity [855].

The Sudan Humanitarian Fund (SHF) has documented prioritised response planning and funding processes for 2026, including a First Standard Allocation of approximately USD 40 million to sustain integrated life-saving health, WASH, and nutrition services and to prioritise national and local partners, indicating active donor-facing reporting and allocation mechanisms [869][867][842]. OCHA guidance and SHF procedures emphasise data protection, informed consent, and careful visibility practices, with SHF operational rules requiring documented proposal review and balanced expert participation, reflecting institutional attention to ethical data-sharing and

governance, though implementation gaps remain [877][842][869].

Qualitative studies of cash programmes highlight persistent information and feedback weaknesses. A December 2025 user-journey study by Ground Truth Solutions and partners found decentralised local responders and strong beneficiary preference for cash, while Collective Sudan (CCS) workshop recommendations (February 2026) call for systematic public information about cash programmes, diversified communication channels, real-time feedback mechanisms, and presence of partner focal points at registration and distribution points to improve two-way information flows [856][841]. Key data points include:

- 33.3 million people requiring humanitarian assistance in 2026 and over 11 million internally displaced [856]
- Over 90% of affected households prefer cash assistance (user-journey study, December 2025) [856]
- 69.3% of households unaware of how to file a complaint; WhatsApp preferred by 78.98% (digital needs assessment) [839]
- SHF First Standard Allocation 2026 approximately USD 40 million to sustain integrated life-saving services [869][867]

Barriers to information

Assessments and qualitative studies found multiple compounding barriers preventing Sudanese populations from accessing aid details and feedback mechanisms [841]. Recipients often lacked basic details — including who the provider was, eligibility criteria, transfer value, and timing — and many first learned about cash assistance via community representatives such as sheikhs and neighbourhood committees, with limited direct engagement with aid providers and minimal post-distribution follow-up [841]. Data were collected in December 2025–January 2026 in Khartoum and North Darfur and findings were validated in February–March 2026 [841]. Centralised distribution points were reported as stressful and hinder access for persons with disabilities [841].

A collapsed financial infrastructure — characterised by high inflation, liquidity shortages, and collapse of formal banking in nearly half the country — combined with administrative and regulatory restrictions on modalities (often permitting only digital transfers) constrains both information flow and modality choice [841][864]. Digital channels and network access are uneven: hotlines and messaging work only where network access permits, and digital delivery may exclude women, older people, and persons with disabilities unless specifically adapted [841]. Language barriers compound these challenges — 21% of Ethiopia-to-Sudan arrivals cited language difficulties and 7% at Eritrea–Sudan crossings — indicating an additional constraint to accessing information and services [886].

Recommended mitigation measures include standardised community information packages, real-time feedback mechanisms (hotlines and staff presence at registration), staggered distributions, and in-person outreach [841].

Communication channels

Primary information channels include WhatsApp, SMS, call-centre systems, mass media, community platforms (community dialogues, religious institutions, schools), household visits, and in-person registration and distribution events, with hotlines and messaging services used where network access permits [839][881][862][841]. Mobile-dependent channels such as bank transfers and mobile wallets are used for cash and voucher delivery and have expanded inclusion where networks and financial services remain functional, but their use is contingent on local network and banking functionality [856][864][841]. Social media and digital platforms are widely used for outreach but also pose misinformation and trust risks that can undermine public health and humanitarian legitimacy [858].

Face-to-face channels and community dialogues are routinely employed for behaviour-change outreach and accountability. In March–April 2026, 10,274 frontline workers, volunteers, and community leaders were trained in interpersonal communication, risk communication, and community engagement to support outreach and feedback collection [881]. Yet awareness of complaint mechanisms remains low across many surveyed households, meaning in-person channels are necessary but not sufficient [881][839].

Key figures on channel preferences among surveyed households:

- 78.98% reported WhatsApp as their preferred feedback channel [839]
- 11.98% preferred face-to-face reporting [839]
- SMS and other mechanisms accounted for under 9% [839]

User-level recommendations from cash-recipient research include systematically informing communities about programmes (provider names, eligibility, transfer values, timelines), diversifying online and offline channels, ensuring staff presence at registration and distribution for real-time feedback, and deploying hotlines and messaging where networks permit to improve timeliness and clarity of information [841].

Misinformation and rumors

Cash recipients in Khartoum and North Darfur reported receiving misinformation from other community members about expected transfer values, eligibility criteria, types of assistance, and payment timelines (interviews conducted in December 2025) [841]. Because recipients commonly first learned about assistance through community representatives such as sheikhs and neighbourhood committees, with limited direct engagement with aid providers, information gaps created mistaken expectations about selection and payments [841][856]. These gaps inhibited household planning and contributed to the likelihood that eligible people remained unaware and excluded from programmes [841].

Coordination and health assessments in 2026 note that community rumours and misinformation are hindering case detection, reporting, and care-seeking in outbreak and health responses, and that information gaps persist across the health system [862][863].

Open-source monitoring and protection analyses show that social media and local commentary can amplify harmful narratives that erode trust in health and humanitarian actors and, in some contexts including Sudan, contribute to hostility or threats against health services and workers [858]. Reported impacts include declining trust, disruption of vaccination and public-health campaigns, and heightened protection risks for health workers and facilities [858].

Recommended mitigation measures include systematic public communication of provider names, eligibility criteria, transfer values, and timelines; diversifying channels and equipping community liaisons with accurate information; real-time feedback and complaint mechanisms such as hotlines and staff at registration and distribution points; and scaling up risk communication and community engagement (RCCE), social listening, and community feedback systems to detect and counter damaging narratives [841][862]. Communities' reliance on local leaders and peer networks for information underscores the need to ensure those representatives receive accurate, timely information to prevent rumour-driven exclusion or misuse of resources [841][858].

Preferred sources of information

WhatsApp is the overwhelmingly preferred feedback channel among affected households (78.98% overall; 59.89% among displaced households), with face-to-face reporting at 11.98% and SMS and other mechanisms accounting for under 9% [839]. Community representatives — including sheikhs, neighbourhood committee members, mutual aid groups, and local responder networks — are primary and trusted sources of information about assistance and registration in Sudan [841][856].

Access to and preference for specific channels varies significantly across population groups. Many women and girls may lack personal phones, share devices, or lack the privacy needed to report sensitive issues, while older persons and people with disabilities often prefer verbal or face-to-face channels [839]. The majority of households (69.3%) do not know how to file a complaint, underscoring the need for repeated, accessible, and trusted communication on available feedback mechanisms [839].

Programmes should therefore diversify and combine channels — including hotlines and messaging services where networks permit, in-person outreach and staff presence at registration and distribution sites, and information materials in local languages adapted for differing literacy levels — to reach different groups effectively [841][881].

Health & Nutrition

Damage to health infrastructure

The Saudi Maternity Hospital in El Fasher was struck multiple times in October 2025 (3 and 7 October), with the 7 October shelling killing at least 13 people, injuring health workers and severely damaging the hospital and its equipment, followed by further attacks on 26–28 October that involved killings, mass patient deaths and abductions of health staff during the city's siege [1011]. Since April 2023, the Rapid Support Forces (RSF) have been linked to the highest number of reported attacks on health care in Sudan, with over 400 recorded cases following the conflict outbreak; monitoring records cite 88 attacks in Darfur between April 2023 and December 2025 and 487 documented attacks on health facilities across Sudan since 2016, as well as 502 reported incidents of military occupation of health facilities [1011]. In early June 2026, intense drone strikes on El Obeid killed dozens of civilians (rights groups reporting at least 23 deaths) and struck residential areas, a funeral gathering and supply trucks, while local hospitals received waves of casualties and medical services were described as increasingly strained [961][966]. In Ghubaysh locality, assessments in June 2026 found the rural hospital – the only facility within a 200 km radius – operating at roughly 10% capacity or less, with 17 PHCs of which two were non-functional and 15 only partially operational, chronic medicine shortages, interrupted cold chain and blood bank services, looted water infrastructure, and staff reduced from 123 pre-conflict to about 70 unpaid or irregularly working personnel, leaving over 500,000 people without free access to nearby healthcare [963]. Khazan Jedeed Hospital's staffing has also collapsed – overall staff reduced from approximately 20 to 4, doctors from 1 to 0, nurses from 3 to 1, laboratory technicians from 2 to 0, and pharmacists from 2 to 0 – with the facility receiving approximately 200 patients per day despite extremely limited medicines and only very basic items available on site [944].

Attacks on health centres

Attacks on health facilities and personnel form part of a wider, documented pattern of violence since the 2023 outbreak of war. On 7 June 2026, a drone strike in Sudri locality (Adeid Raha area) hit two civilian vehicles carrying people travelling to seek medical treatment, killing three and injuring another, illustrating how attacks on transport routes are directly impeding access to health services [933][961]. Separately, drone strikes on 7–8 June 2026 in El Obeid, North Kordofan, killed up to 23 civilians and hit residential neighbourhoods, a funeral gathering, and vehicles including a food truck and a fuel station, further disrupting civilian access to care [961][966][933]. These incidents reflect a broader, systematic pattern: health facilities have been repeatedly raided, occupied or shelled since the conflict began, including the repeated assaults on the Saudi Maternity Hospital in El Fasher in October 2025 that caused multiple deaths, injuries, facility destruction and abductions of health workers [1011].

Availability and cost of medicines and health nutrition services

Across assessed areas, availability of medicines is critically limited. At many facilities, stocks are restricted to basic items such as malaria RDTs, paracetamol and limited Coartem; patients are routinely asked to purchase drugs from local markets; and therapeutic nutrition supplies are scarce, with one pharmacist reporting only approximately 20 boxes of Plumpy'Nut on site [944]. The lone functioning clinic in Sirba lacks diagnostic toolkits, protective gear and basic antibiotics, prompting some families to cross into Chad to purchase medicines [941]. UNICEF-supplied vaccines to Al Fula have not arrived for the past three years, and WHO's most recent delivery to Ghubaysh arrived approximately one month before 24 May 2026 after a six-month gap [963]. High costs are further blocking access: soap costs 1,500–1,600 SDG, placing hygiene items out of reach for many households [963][944]. These shortages stem from major drug-supply disruptions – road closures, destroyed warehouses and import constraints – creating near-total dependence on external humanitarian supply lines [965]. Logistics disruptions at regional hubs have further delayed shipments, with the International Rescue Committee reporting one essential medical supply shipment held for two months in Dubai [993]. To address these gaps, the Sudan Humanitarian Fund (SHF) First Standard Allocation 2026 explicitly prioritises uninterrupted medical supply chains and allocates USD 20.5 million for health interventions and USD 6.5 million for nutrition, including procurement and last-mile distribution of essential medicines, RH kits and SAM kits [945][950].

Impact of the conflict on doctors and health staff

The conflict has had a severe and sustained impact on health personnel across Sudan. Between 2016 and 2025, more than 3,600 health workers were reported killed, 1,558 kidnapped, and 2,575 arrested, contributing to long-term loss of medical personnel and services [1011]. Security incidents have continued into 2026: in early June, RSF arrest campaigns in West Darfur reportedly included employees of international organisations – including two staff from Doctors Without Borders and CRS among detainees reportedly transferred to Nyala – constraining staff safety and humanitarian operations [1010]. At the facility level, staffing has collapsed at key hospitals. Ghubaysh Rural Hospital, the only facility within a 200 km radius serving over 500,000 people, had 123 staff before the conflict and retains approximately 70, many of whom are unpaid and not working regularly [963]. The cholera outbreak in West Kordofan has further strained remaining health workers, who are managing emergency CTCs with minimal resources [963].

Disease outbreaks

Recent assessments in early–mid June 2026 report multiple concurrent outbreaks across western and central Sudan amid severe WASH failures and near-collapse of local health services [930][963][928]. In AnNuhud (West Kordofan), a suspected cholera outbreak had rapidly escalated to over 300 cumulative cases and more than 70 deaths by late May 2026, with an alert issued 19 May 2026 and 345 cases and 96 deaths (CFR 27.8%) reported by 2 June 2026 [930][931][963]. No rapid diagnostic tests were available domestically, so samples had to be sent to Abyei and Juba for confirmation, and emergency cholera treatment centres were stood up by partners [963]. A rapid multisector assessment in Ghubaysh (West Kordofan) conducted 19–25 May 2026 found ongoing AWD/cholera and measles outbreaks, with the main Ghubaysh hospital operating at minimal capacity and severe shortages of medicines and supplies

[928][963]. In Riteiret (Ghubaysh locality), the MoH and assessment teams confirmed 40 measles cases on the day of visit; communities reported 15 deaths in the preceding two weeks, mostly children aged 5–10, with approximately 90% of measles cases severely acutely malnourished and no children in the locality having received measles vaccination due to undelivered stocks. Solidarités International teams began immediate response actions including water chlorination, establishment of an isolation tukul, PPE provision and community awareness [963]. Across assessed IDP sites and camps, teams reported cholera and measles transmission, near-total absence or dysfunction of latrines (observed 1 latrine per 87 households), open defecation, unaffordability of soap, and widespread diarrhoeal disease, malaria and malnutrition driven by WASH and service gaps [944][963]. In Central Darfur (Jebel Marrah), a suspected mpox outbreak was estimated at about 80 cases as of mid-May 2026 [930][931]. In Sirba locality (West Darfur), a meningitis epidemic reportedly killed more than 90 people, mainly children, in an area described as effectively cut off from international assistance [941]. Humanitarian analyses warn these outbreaks are likely to escalate through the rainy season given very poor WASH access, overcrowded IDP sites, interrupted vaccination and weakened health system capacity, raising acute malnutrition and mortality risks [930][931][963].

Health and nutrition needs and priorities

Rapid multisector assessments conducted in May–June 2026 across Khazan Jedeed (East Darfur) and Ghubaysh (West Kordofan) documented severe gaps in basic services, critically degraded health and WASH infrastructure, near-minimal hospital capacity, and urgent needs spanning health, nutrition, WASH, shelter and food security [929][928][963][944]. Communities consistently ranked health, food, water and shelter among their top priorities, reporting very limited access to medicines, widespread reduction in meal frequency (often one meal per day), high acute malnutrition including multiple SAM referrals, and child deaths linked to severe malnutrition [944]. No malnutrition treatment centre or nutrition supplies were present in assessed areas of West Kordofan, and measles vaccination campaigns had not taken place [928][963]. In response, Solidarités International implemented water trucking, handpump rehabilitation and emergency SNFI distributions (including approximately 1,100 households) in Khazan Jedeed in May 2026, while ICRC expanded air links between Port Sudan and Khartoum to accelerate deployment of specialised personnel and sustain operations closer to affected communities [944][984]. The Sudan Humanitarian Fund (SHF) First Standard Allocation 2026 has allocated USD 40 million to restore and sustain integrated, life-saving health, WASH and nutrition services in the highest-severity and hardest-to-reach localities, with a strong emphasis on national and local partners as primary recipients [967][950]. SHF guidance specifies maintenance of a minimum PHC/SRH package, emergency and trauma care, disease surveillance and outbreak response (cholera, measles, hepatitis E, dengue, malaria), infection prevention and control, MHPSS integration, and intersectoral referral pathways [945]. The combined operational priority is to keep health facilities open and functioning by integrating WASH and nutrition support into facility-based and community health service delivery, strengthening outbreak preparedness and response, and scaling localised partner capacity and referral systems [967][950][945].

Education

Impact of the crisis on school infrastructure and personnel

Field assessments report that education services in many conflict-affected and displacement settings are largely non-functional due to absent teacher incentives, shortages of teaching materials, lack of seating and basic infrastructure [1047][1068][1046]. Schools in Ghubaysh only reopened in May 2026, while many gathering and IDP sites occupy former schools that stopped functioning since the start of the conflict [1046]. On 8 June 2026, roughly 10,000 students sat Sudanese Certificate examinations in Darfur and parts of Kordofan under arrangements by the RSF-aligned Founding Alliance, conducted across approximately 84 centres in eight states [1041][1055]. Female students constituted about 74% of examinees in those locations, and the opening day in Nyala was marked by heightened security measures, including federal police vehicles deployed around examination centres [1055][1041]. Authorities in areas under the Founding Alliance have also announced plans to rehabilitate universities and expand access to higher education [1041]. UNICEF and other observers warned that the emergence of parallel examinations risks deepening political and institutional divisions and may jeopardise recognition of qualifications [1034][1041].

Access to education

Conflict, displacement, and damage to infrastructure have left many schools closed or non-functional across Sudan. Large numbers of IDP children have dropped out because families cannot afford school-related costs, estimated at approximately 7,000 SDG per year in some locations and reported at 6,000 SDG per month in others [1046][1047]. Movement restrictions, ambushes, drone strikes, scarcity of vehicles, and high transport costs have made travel to services and evacuation dangerous and costly, further constraining students' ability to reach schools or examination centres [1055][1064]. The wider displacement context adds significant pressure on education: the Sudan crisis is associated with over 1.2 million refugees and asylum-seekers, and Sudan hosts significant refugee caseloads including 67,062 Ethiopian refugees as of 31 May 2026, increasing demand on already limited education services [1074][1073]. Displacement-affected populations are disproportionately women and children, heightening risks of school dropout and barriers to attendance for these groups [1046].

Education needs and priorities

Assessment missions in East Darfur (Khazan Jedeed) and West Kordofan documented major education needs, including lack of school infrastructure, high school fees, and closed or non-functional schools, with a call to re-establish school facilities to enable safe return to learning [1036][1047][1046]. The same assessments highlighted critical gaps: limited humanitarian presence and response capacity, rapidly growing displacement increasing pressure on local infrastructure, and outdated or incomplete mapping of gathering sites that risks excluding children in non-official sites from education assistance [1036][1047][1046].

On 10 June 2026, UNICEF publicly urged that all Sudanese students be allowed to sit the same nationally recognised examinations, offering to support practical arrangements to administer unified exams safely and consistently nationwide, in order to protect learners' access to certification and future opportunities and prevent long-term fragmentation of qualifications [1034][1035]. Assessment recommendations emphasise advocacy to education actors to re-establish school facilities, deploy alternative and community-based learning where formal schools are closed, and integrate protection and mental health/psychosocial support within education settings to address barriers to attendance and learning continuity [1047][1046][1036]. Regional coordination for education support to returnees is ongoing, with IOM coordinating with UNICEF, UNFPA, WFP, and international and national NGOs to deliver education assistance alongside other services in returnee sites in Chad as part of the wider Sudan crisis response [1071].

WASH

Damage to water infrastructure

A rapid assessment in Ghubaysh (19–25 May 2026) found a single private water yard (50 m³ tank, drilled 2018, 5.5 kW pump) with an underperforming solar system (32 panels, 8 malfunctioning), frequent pump failures, broken collection points, multi-hour queues, and high user fees (approximately 2,000 SDG per barrel) [1083][1084]. In Khazan Jedeed, assessments in early May documented ongoing water trucking and handpump rehabilitation needs for three IDP camps, noting insufficient clean water, active water trucking operations as of 6 May 2026, and prior cholera outbreaks in 2025 linked to degraded WASH infrastructure [1082][1085]. Explosive weapons and drone strikes have damaged civilian infrastructure and water systems across Sudan; a drone strike on the Ardamata Bridge on 10 June 2026 halted a key aid corridor into West Darfur, impeding delivery of relief items including water supplies [1130][1094][1099]. On 11 June 2026, experts warned that weakening institutional capacity at the Ministry of Irrigation and Water Resources threatens the long-term protection, operation, and rehabilitation of Sudan's water infrastructure, compounding immediate damage and recovery challenges [1088]. Widespread degradation of water and sanitation systems across Sudan has resulted in heavy reliance on water trucking and point sources, contributing to severe shortages that place an estimated 1.1 million people in Southern Darfur at heightened risk of cholera and other waterborne diseases [1105][1084].

Access to clean water

In Al Nahil IDP camp, a Solidarités International (SI) assessment on 6 May 2026 found approximately 18,020 people relying on a single manual handpump as the only permanent water source, with high salinity and suspected contamination leading households to avoid using it for drinking or cooking and prompting collection of a water sample for analysis [1082]. Water trucking had previously supplied the camp via two bladders at approximately 150 barrels/day (≈30 m³) for roughly two months before operations stopped, after which households became dependent on purchased water at reported prices of 4,000 SDG per barrel and 200 SDG per jerrycan [1082]. In Khazan Jedeed, water supply is largely privately managed with roughly 45 handpumps in town (31 functional, 14 needing rehabilitation) and additional functionality gaps in the wider locality (13 non-functional, 10 functional), constraining access for displaced and host populations [1082]. A rapid SI assessment in Ghubaysh (19–25 May 2026) documented 12 water yards (five government, remainder private) operating 12–16 hours a day but overused and facing storage and leakage losses, with water drawn from boreholes at depths of 700–800 feet that is generally slightly saline and not chlorinated, and people queuing 2–3 hours or travelling up to 6 hours round trip to fetch water [1083][1084]. Households in Ghubaysh relied on purchased water at reported prices of 1,500 SDG per drum and 150 SDG per 20L jerrycan, while water trucking was deemed an inappropriate long-term response to avoid dependency [1083]. At the national response level, a Port Sudan meeting on 8 June 2026 reviewed the Port Sudan Water Project—implemented by UNICEF and funded at \$51 million by the African Development Bank—to enhance drinking water services in Port Sudan [1089]. The SHF First Standard Allocation 2026 (USD 40 million) prioritises WASH interventions, allocating funding for WASH in health-care facilities, cholera/AWD preparedness and response, chlorination and water-quality monitoring, and short-term water trucking to bridge gaps to semi-permanent solutions, with a WASH envelope of \$13,000,000 [1092][1103]. UNICEF continues to deliver safe drinking water to locations including Kosti, Sindaiti and the Mountain (approximately 15,500 people reached), while an estimated 1.1 million people in Southern Darfur face severe water and sanitation shortages and heightened cholera risk [1105].

Contamination of water sources

Municipal solid-waste services have largely stopped in conflict-affected areas such as Ghubaysh (West Kordofan), with waste commonly disposed of at open sites and SI water-quality testing confirming contamination that requires chlorination [1083]. No desludging activities have taken place for years, latrines need rehabilitation, medical waste management is inadequate due to a non-functional hospital incinerator, and Ghubaysh Rural Hospital lacks chlorine and PPE, worsening infection risks [1083]. The hospital's borehole and water tank are non-functional, and the main pipeline fills the tank only approximately two hours per week—insufficient for health-care needs and continuity of safe water supply [1083]. Similar gaps in solid-waste management were documented for El Fasher, with recommendations to rehabilitate final disposal sites and train municipal staff, noting increased vector proliferation and pollution risks where municipal systems have collapsed [1082]. Environmental surveillance detected vaccine-derived poliovirus type 2 (cVDPV2) in sewage in 2026 (three sewage samples from Sudan), and one cVDPV2 case was reported in Sudan in 2026, indicating ongoing fecal-oral transmission risk linked to sewage contamination and poor sanitation [1149]. Sudan experienced a large cholera/AWD outbreak in 2025, with 71,825 suspected cases and 1,194 deaths across 55 of 80 counties, underscoring the severe public-health consequences of WASH system failures [1128]. Humanitarian planning in 2026 (SHF) prioritizes water-quality monitoring and chlorination across the water chain, functional sanitation and health-care waste management, and targeted cholera/AWD/Hepatitis E response activities to address these contamination and disease risks [1103]. Limited community outreach has been reported locally—such as Friday-prayer mosque awareness sessions in Ghubaysh—but overall risk communication, community engagement, and vector-control measures remain insufficient in affected areas [1083].

Water needs and priorities

Rapid assessments and site visits report severe deficits in water quantity and quality, widespread open defecation, and almost non-existent sanitation and bathing facilities across multiple displacement sites [1082][1083]. In Al Nahil IDP camp, assessors found roughly 60 latrines for over 18,000 people (approximately 1 latrine per 300 people), no bathing facilities, and only a single hygiene-kit distribution targeting approximately 120 households in early 2026, leaving coverage extremely limited [1082]. High market prices make hygiene items unaffordable (soap 1,500–1,600 SDG), and limited water access forces communal sharing of jerrycans (costs of approximately 500 SDG per jerrycan), undermining handwashing and safe storage practices [1082][1083]. A suspected cholera outbreak in An-Nuhud (West Kordofan) recorded over 300 cumulative cases and more than 70 deaths by late May 2026, with poor WASH access identified as a key driver [1096]. Assessment teams in Ghubaysh (West Kordofan) identified ongoing AWD/cholera and measles outbreaks and recommended emergency water trucking, repair and rehabilitation of handpumps and water systems, provision of household water treatment chemicals, desludging, latrine construction, and community hygiene promotion including menstrual kits [1084][1083]. The Sudan Humanitarian Fund's 2026 allocation strategy prioritises sustained water trucking as an immediate measure and medium-term investments in boreholes, water yards and distribution networks, construction and rehabilitation of latrines, health-facility water supply improvements, chlorination, and targeted WASH NFI distributions for cholera response, with a dedicated WASH cholera/HCF package and an identified ~\$13 million facility-focused WASH line [1103][1092]. IOM reported that regional response activities for Sudan-affected returnees in Chad delivered improved water and sanitation to 223,969 people through 17 boreholes, 7,836 WASH kits, and 630 latrines, demonstrating scalable interventions in neighbouring areas [1113]. Key operational constraints hampering scale-up include limited humanitarian presence in hard-to-reach localities, logistical and fuel constraints, insufficient storage and treatment points, and funding shortfalls that have forced the cessation or reduction of water trucking in some areas [1083][1092][1107].

Sanitation & Hygiene

Access to adequate sanitation facilities

A community-level digital needs assessment found 56.93% of latrines reported as not functional, with cleanliness (15.33%), privacy (8.03%), safety (10.22%), and limited accessibility (3.65%) also noted as major issues, indicating widespread unusable sanitation infrastructure rather than pure lack of coverage [1158]. In Al Nahil IDP camp, roughly 60 latrines were serving over 18,000 people (approximately 1 latrine per 300 people), well below the SPHERE minimum of 1:20 and linked to widespread open defecation and high public-health risk; only 13 latrines had been constructed by World Vision in the camp [1157]. Assessments in West Kordofan (Ghubaysh) conducted 19–25 May 2026 observed near-total absence or severely degraded latrines in displacement sites — Al Itilad had only four latrines (requiring desludging, rehabilitation, and doors) and Al Khanza had one latrine observed — with open defecation reported as common practice [1156][1177]. Severe water and sanitation shortages are estimated to affect 1.1 million people in Southern Darfur, exacerbating cholera and other disease risks [1173]. As part of cross-border interventions in the Chad response to the Sudan crisis, IOM reported the construction of 630 latrines and related WASH assistance that improved sanitation access for 223,969 returnees and host-community members [1195].

Prevalence of WASH coping mechanisms

Women and girls in West Kordofan displacement sites reported waiting until early morning or after dark to relieve themselves due to the near-total absence of functional latrines and lack of privacy [1156]. In Al Nahil IDP camp, the severe latrine shortage (approximately 1 per 300 people) drives widespread open defecation, use of ashes or sand instead of soap, and infrequent bathing (about once every three days) [1157]. Households across assessed sites reported being unable to afford soap at market prices of 1,500–1,600 SDG, further limiting hygiene options [1157]. Displaced households rely heavily on emergency and costly water sources, including water trucking (39.86%) and bottled water (22.46%), while public networks serve only 24.64% of households [1158].

Availability and cost of hygiene items

Assessments in early June 2026 found hygiene items extremely limited in displacement sites, with soap generally only available for purchase at approximately 1,500–1,600 SDG — unaffordable for most households [1157][1156]. Humanitarian hygiene support has been minimal: one hygiene-kit distribution targeting roughly 120 households was reported at the beginning of 2026, while other sites reported never receiving hygiene kits or soap distributions [1157]. As a consequence, residents commonly wash without soap or use ashes and sand as substitutes, and bathing is infrequent (about once every three days or once–twice weekly) due to limited water and lack of dedicated bathing facilities [1157]. Voucher and market restrictions also limited effective access to hygiene items for some recipients, and community assessments highlighted unmet needs for menstrual hygiene materials, dignity kits, and other hygiene NFIs for women and girls [1217][1158].

WASH driven disease risks

Extremely poor access to WASH services is exacerbating multiple disease outbreaks and likely worsening already-high acute malnutrition across Sudan [1163][1165]. UNICEF issued an alert on 19 May about suspected cholera in Fuja (Al Fula locality); an assessment conducted on 3 June 2026 reported cases had rapidly risen from initial reports of 96 cases and 27 deaths to 345 cases and 96 confirmed deaths (CFR 27.8%) by 2 June, with cholera treatment centres operating in Fuga and Um Nuhud and WASH partner CAFA and health partner Alight leading the response while samples are sent to Abyei and Juba due to the absence of RDTs in-country [1156]. A suspected Mpox outbreak in Central Darfur's Jebel Marrah was estimated at approximately 80 cases by mid-May, raising additional contagion concerns [1163][1165]. Overcrowded IDP settlements — notably Tawila in North Darfur, which hosts over 700,000 IDPs — are considered high-risk settings for further spread of waterborne and other contagious diseases [1163][1165]. Community surveys report major water quality and quantity problems: 52.9% of sampled people said water quantity is insufficient, and displaced respondents frequently report taste, color, and visible impurities as indicators of contamination [1158].

SMART surveys from the November–January harvest/post-harvest period already showed GAM above the Critical threshold (15%) in multiple locations — Meiram 20.5%, Muglad 21.2%, Gebrat Al-Sheikh 16.3%, Al Koma 18%, Kutum 24.3%, Saraf Omra 15.1% — and acute malnutrition is expected to worsen through the lean season amid poor WASH conditions and deteriorating health services [1163][1165]. The Sudan Humanitarian Fund's 2026 First Standard Allocation includes approximately \$13,000,000 for a dedicated WASH cholera/AWD outbreak response package and approximately \$20,500,000 for maintaining health service continuity and outbreak preparedness [1162][1172].

Sanitation and hygiene needs and priorities

Recent assessments describe critical sanitation and hygiene gaps marked by widespread open defecation, non-functional latrines, lack of dignity and privacy features for women and girls, and unaffordable hygiene commodities, raising acute public-health and protection risks [1156][1157][1177][1158]. Severe WASH deficits are linked to disease outbreaks, including a suspected cholera surge in An-Nuhud (West Kordofan) with over 300 cumulative cases and more than 70 deaths as of late May, heightening urgency for cholera-focused WASH interventions [1165][1163]. Humanitarian deliveries have included a UNICEF-led WASH convoy (eight trucks to Dilling and Kadugli) supporting nearly 39,000 people, and UNICEF/OCHA-reported safe water provision to approximately 15,500 people in Kosti, Sindalti, and the Mountain, though needs remain far larger given severe water and sanitation shortages affecting an estimated 1.1 million people in Southern Darfur [1186][1173]. The Sudan Humanitarian Fund's 2026 First Standard Allocation — part of a broader \$40 million allocation strategy — prioritizes WASH with \$13,000,000 dedicated to WASH activities, integrating cholera/AWD preparedness and response, sustained water trucking, latrine construction and rehabilitation, and targeted WASH-NFI distributions to health facilities, camps, and affected households [1162][1176].

Food Security

Availability and cost of food

Markets remain generally functional in many localities but are highly fragile, with affordability and resilience rated below average in nearly two-thirds of monitored areas [1243][1240]. Joint market monitoring identifies purchasing power — not supply availability — as the binding constraint for households, with affordability the main barrier in roughly 70% of assessed markets (38 of 54) [1231]. Specific markets were identified with poor scores across availability, accessibility, affordability, resilience and infrastructure — notably Dilling (South Kordofan), As-Sunut (West Kordofan), Nyala Shimal (South Darfur) and Kereneik (West Darfur) [1243].

Food prices rose sharply in April 2026 (sorghum +23% month-on-month; wheat +17.5% month-on-month; millet +14%), with staple prices remaining far above five-year averages, driven by seasonal stock declines, conflict-related supply disruptions, and currency and fuel shocks [1240]. Fuel shortages and price spikes in April 2026 (gasoline +15%, diesel +55% month-on-month) are increasing transport costs and widening regional price disparities, worsening market access costs for traders and consumers [1240]. Drone attacks, road degradation and other conflict-related disruptions have damaged trade routes and markets and are expected to persist through the rainy season, increasing risks of commercial and humanitarian access deterioration and expanding Emergency/IPC-5 risk in North Darfur and South Kordofan [1240][1243].

Cash and multipurpose cash assistance (MPCA) are reported to restore access where markets function, with post-distribution monitoring showing rapid food security gains, reductions in negative coping, high delivery satisfaction, and receipt rates of expected cash values of 97–100% [1231][1254][1253]. Humanitarian scale remains constrained: WFP reached 3.1 million people with food, cash and nutrition assistance in April 2026, but funding shortfalls, supply disruptions and insecurity limit coverage and the ability to meet the planned target of approximately 4.4 million beneficiaries between June–September [1243].

Coping mechanisms

Households across displacement sites report severe short-term coping strategies. In Khazan Jedeed/Al Nahil IDP camp, many consume only one to two meals per day and rely on food-sharing, while communal kitchens operate only intermittently (last conducted in March 2026); targeted cash interventions by CRS (approximately 700 households) and World Vision (17 households) have not eliminated acute needs, and severe child malnutrition-related deaths have been reported [1242]. In Ghubaysh (West Kordofan), households mostly consume one meal per day, food distributions are non-existent, communal kitchens are irregular, dietary diversity is extremely limited, and MPCA is absent — conditions that drive reliance on negative coping strategies and external support [1239][1244].

Post-distribution monitoring from CCS and WHH demonstrates that multipurpose cash assistance rapidly improves food security and reduces negative coping where markets are accessible: the proportion of households consuming one meal or less fell from 18.6% to 0.6%, Acceptable Food Consumption Score rose from 29.4% to 55.2% while Poor FCS dropped from 43.2% to 16.2%, and the share of households reporting high reliance on negative coping fell from 31.1% to 13.0%, with low-coping households rising to 43.6% [1231].

Households are also using mobility as a food-security coping strategy: cross-border monitoring indicates movements are predominantly individual and that over half of individuals returning from Ethiopia cited economic or livelihood opportunities as their primary reason for return (63%) [1273]. Humanitarian assistance remains insufficient in scale, leaving many households dependent on inadequate aid and negative coping strategies [1243].

Food security needs and priorities

Humanitarian assessments identify livelihoods and food as the top priorities for affected households, with livelihoods cited by 47.12% of respondents and food typically ranked second across displacement statuses [1230]. Food stocks are highly limited among displaced populations, with 74.06% of displaced households reporting food reserves lasting up to seven days [1230]. Food assistance needs differ by living arrangement: collective shelters require support for collective meal distributions, cooking capacity, storage and kitchen items, while households outside shelters need food parcels and flexible support to meet household-level needs [1230].

At national and regional levels, FEWS NET projects that around 40–45% of Sudan's population will need humanitarian food assistance, with Emergency (IPC Phase 4) outcomes expanding and credible risk of Famine (IPC Phase 5) in parts of North Darfur and South Kordofan [1232][1243][1240]. WFP reached 3.1 million people with food, cash and nutrition assistance in April 2026 but remains far below estimated needs, with WFP distributions in Khazan Jedeed/AI Nahil covering roughly 15% of the camp (approximately 501 targeted households) and no cash assistance reported there [1242].

Shelter & NFI

Damage to housing and residential infrastructure

Rapid assessments in late May–early June 2026 document widespread shelter inadequacy, makeshift housing, and severe pressure on local housing stock across multiple Sudan states. In West Kordofan's Ghubaysh locality (assessment 19–25 May 2026), more than 22,153 IDPs were identified, most (about 69%) living within host communities under overcrowded conditions with no sustained humanitarian shelter response planned ahead of the rainy season [1282][1295]. The Ghubaysh city population had risen to approximately 30,000–40,000 (from ~20,000 pre-war) and the wider locality to ~250,000 (from ~150,000), sharply increasing demand on already limited housing and services [1282]. Site-level estimates show AI Iltihad ground site hosting approximately 300 households (~2,000 people) and AI Khansa ground site hosting approximately 150 households (~1,500 people); both sites report ~95% of residents are women and children, shelters are handmade and highly exposed to wind and rain, and no households possess proper tarpaulins [1282]. In East Darfur (Khazan Jedeed), a May 2026 assessment identified critical multisectoral needs and increasing pressure on local infrastructure, with shelter needs driven by continued population influx and limited humanitarian presence [1283]. The AI Nahil IDP camp assessment (5 May 2026) found the majority of shelters to be makeshift structures of wood, dry grass, and salvaged materials, with no households possessing tarpaulins and shelter conditions critically inadequate ahead of the rainy season; a fire on 14 May 2026 reportedly affected roughly 400 households, and the last distribution of shelter materials in the camp had occurred in June 2025 [1281]. Overall, reports highlight acute gaps in emergency shelter and NFI coverage, high dependence on host families, exposure to environmental hazards, and limited planned shelter distributions — factors that impede safe returns, prolong displacement, and complicate rebuilding amid widespread destruction, looting, and abandonment of housing in affected areas [1282][1281][1283][1316].

Camp and displaced populations

Rapid multi-sector assessments in Khazan Jedeed (East Darfur) and Ghubaysh (West Kordofan) in May–early June 2026 found displaced people living in severely precarious, overcrowded sites with minimal sustained humanitarian presence and rapidly worsening public-health risks [1283][1295]. Key population figures from assessed camps include: AI Salam hosting 9,555 individuals; AI Nahil hosting 18,020 individuals; AI Wahida hosting 18,000 individuals — totalling approximately 45,575 people (~8,000 households) [1281]. Humanitarian estimates indicate more than 80,000 IDPs concentrated in the wider area, and the Khazan Jedeed administrative unit may host several hundred thousand displaced people [1281]. Shelter conditions are overwhelmingly makeshift — constructed from branches, straw, and fabric — providing minimal protection from weather and little privacy [1281]. Water access is critically below emergency thresholds with almost no functioning water source inside camps, and sanitation coverage is extremely insufficient (approximately 1 latrine per 87 households), with widespread open defecation observed across gathering sites [1281][1282]. Soap and basic hygiene items are unaffordable for many households (soap prices reported at 1,500–1,600 SDG), further increasing disease risk [1282]. Acute health risks were documented: cholera/acute watery diarrhoea (AWD) and measles outbreaks were confirmed in displacement sites, and shortages of medicines and IV fluids have been linked to confirmed deaths in assessed locations [1281][1282][1295]. Food security is severely degraded — households report consuming one meal per day, food diversity is absent, and distributions reach less than half of camp populations, with recent arrivals often excluded [1281][1282]. The demographic profile of assessed sites is dominated by women and children, with men largely absent. Documented protection concerns include family separation, unaccompanied and separated children, child-headed households, cases of sexual violence against girls, and a lack of assistive devices and adapted services for persons with disabilities [1281]. People arriving at sites also reported incidents during transit, including harassment, looting, and physical and sexual abuse [1282]. The combination of extreme deprivation, overcrowding, poor WASH conditions, and limited health and protection services substantially heightens protection and public-health risks across all assessed displacement sites [1281][1282].

Shelter NFI needs and priorities

Critical NFI items reported as urgent needs across assessed sites include blankets, mattresses, mosquito nets, kitchen/cooking utensil sets, hygiene kits (including menstrual health management kits and diapers), jerrycans, buckets, tarpaulins, and mats for household use and shelter reinforcement [1280][1304][1281]. High prices and limited availability of NFIs on local markets are repeatedly cited as a primary barrier to access and a protection risk — particularly exposure to mosquitoes and lack of cooking materials — with affordability rather than absolute market failure identified as the main constraint in many areas [1281][1282][1295][1305]. Assessments across Khazan Jedeed (5 May 2026) and Ghubaysh (19–25 May 2026) consistently highlight limited humanitarian coverage relative to needs and high dependency on external assistance, underscoring the urgent need to scale up NFI and shelter interventions [1281][1283][1295]. Recommended priority actions include in-kind NFI and shelter-kit distributions, provision of safe and dignified transitional shelter, installation of overhead solar lighting around sanitation areas, and distribution of household hygiene and menstrual kits [1281][1282][1295]. In terms of response reach, IOM Chad reported reaching 128,211 returnees with shelter/NFI interventions — including 5,310 transitional shelters built and 27,525 NFI kits distributed — and supporting 367,757 people with multisectoral assistance as of May 2026 [1320]. In the Central African Republic response (3–9 June 2026), partners distributed 171 NFI kits to 171 households (approximately 850 people) containing blankets, kitchen sets, mosquito nets, buckets, jerrycans, tarpaulins, and mats [1304][1311]. The Sudan Humanitarian Fund's First Standard Allocation 2026 earmarked USD 40 million to sustain life-saving services, with sector lines including approximately \$20.5 million for health continuity and \$13 million for WASH interventions — the latter

specifying targeted WASH NFI distributions and ring-approach outbreak responses [1294][1319]. Logistics and access constraints are significantly hampering delivery: attacks that damaged the Ardamata Bridge (reported 13 June 2026) disrupted a key West Darfur corridor and threaten humanitarian supply movements, while broader fuel and supply-chain disruptions hamper in-kind distributions across affected states [1331][1305]. Evidence cited by sector reviews indicates that cash (MPCA) is cost-effective and can achieve shelter/NFI outcomes where markets function and purchasing power is the binding constraint; however, administrative restrictions and financial access requirements may limit cash options in some areas, underscoring the need for mixed-modality planning [1325][1305].

Livelihoods & Agriculture

Impact on livelihoods and household incomes

The conflict has severely disrupted Sudan's economy, causing high inflation, currency depreciation, liquidity shortages, fragmentation of the financial system, and the collapse of formal banking in nearly half the country [1336][1332]. Conflict-driven displacement — with 8.9 million internally displaced people as of May 2026 — and an expanding food crisis are eroding income sources and access to food across Greater Darfur, Greater Kordofan and Blue Nile, worsening livelihoods during the lean season [1347]. Oxfam's Digital Needs Assessment found households reporting widespread loss of income and an affordability crisis affecting rent, food, household items (including baby supplies and medication), transport and other essentials, with livelihoods emerging as the top priority in April 2026 [1343]. Markets at the state level remain largely functional, but cash and voucher assistance is the preferred form of support, with over 90% of affected households identifying cash as the most dignified form of assistance [1332].

Barriers to farming and agriculture

Conflict, insecurity and attacks on infrastructure have restricted farmers' safe access to land, markets and supply routes. Drone strikes and the destruction of bridges and roads in June 2026 disrupted commercial and humanitarian access across Darfur and Kordofan, while drone attacks targeting markets and trade routes also persisted in May 2026 [1363][1347][1342]. High and rising fuel and fertilizer costs, tight fuel supplies and sharp staple price increases since April–May 2026 have raised production and transport costs and squeezed farmers' ability to purchase inputs [1347][1357][1394][1368]. Assessments also report shortages and unaffordability of seeds, tools and rented land [1380][1375][1359]. Major irrigation schemes and related infrastructure have been degraded or neglected after years of conflict, with looted machinery, destroyed research stations and blocked drainage undermining irrigation plans and productivity [1355]. Institutional capacity to support agriculture has been further weakened by the migration of qualified irrigation cadres, reluctance of professionals to work in reorganized ministries, and administrative changes that have eroded water- and irrigation-management expertise and operational capacity [1392]. Humanitarian and development responses aimed at restoring production and inputs face persistent access, market and funding constraints on the ground [1360][1370][1373].

Livelihood agriculture needs and priorities

On 8 June 2026, Finance Minister Dr. Gebreil Ibrahim chaired a high-level meeting with AfDB, WFP and others to secure urgent financing for the 2026–2027 summer agricultural season, with the Ministry of Agriculture presenting immediate input needs — including improved seeds, fertilisers, fuel and pesticides — for strategic crops such as sorghum and millet [1359]. Sector assessments also flag urgent needs for rapid financing and rehabilitation of irrigation, drainage and water-management infrastructure, plus concessional finance to enable land preparation, mechanisation and input access ahead of the season [1355][1348]. The African Development Bank and WFP launched an \$87 million BOOST project on 22 May 2026 to strengthen agrifood resilience in rain-fed areas (Sennar and Blue Nile), targeting over 230,000 farming households to restore production, reduce post-harvest losses and increase cereal and pulse output over four years, in a context where more than 19 million people face crisis-level hunger [1370][1360]. FAO reported vaccination of more than 6.2 million livestock across 17 states — protecting the livelihoods of roughly 1.9 million people — and emphasised supporting farmers, herders and fishers as a critical livelihood intervention [1394]. Cash-based assessments recommend scaling Multi-Purpose Cash Assistance (MPCA) and cash-based livelihood support where markets operate, as household purchasing power is identified as the primary constraint to food and livelihood recovery [1373][1362].

Protection

Attacks on civilians

Repeated drone and aerial strikes in Kordofan in early June 2026 caused significant civilian deaths and injuries, with the Emergency Lawyers Group reporting at least 33 civilian deaths in North Kordofan in the seven days before 8 June 2026 and local monitors documenting dozens of strikes between 6–8 June 2026 that killed and wounded civilians [1442]. The Abu Zaima market and surrounding villages in Hamrat El Sheikh were struck on 6–7 June 2026, with at least 15 civilians killed and dozens injured [1442]. On 7 June 2026, drones struck two civilian vehicles in Adeid Raha (Sudri locality), killing three civilians travelling for medical treatment, and a separate drone strike on the Charter fuel station in El Obeid triggered a large explosion that killed at least one civilian and damaged essential infrastructure [1442]. A separate May 2026 strike on the Ghubaysh market killed at least 28 civilians; Ghubaysh has since become a major displacement and humanitarian hotspot with IDP estimates ranging from 22,153 (DTM, 12 April 2026) to local authority figures of 68,000 and an SI assessment closer to 30,000 [1412]. Monitoring groups and rights organisations have described these strikes as part of a recurring pattern targeting markets, civilian vehicles and service infrastructure, and have called for independent investigations and accountability [1442].

Attacks on healthcare and medical infrastructure have been widespread, including repeated assaults on hospitals and maternity services in Darfur — with 88 attacks recorded between April 2023 and December 2025 and multiple deadly attacks on El Fasher hospitals in October 2025 — further degrading access to lifesaving care [1441]. Humanitarian analysis warns of expanding Emergency/IPC Phase 4 outcomes and localized risk of Catastrophe/IPC Phase 5 in North Darfur and Greater Kordofan, driven by conflict, drone attacks on markets and trade routes, early depletion of food stocks, and disease outbreaks [1400].

Recent multisector assessments in West Kordofan and East Darfur report near-total absence of sustained humanitarian response, critical gaps in WASH and health services, active AWD/cholera and measles outbreaks, severe food deficits with households reporting one meal per day, and collapsing market and supply chains that compound protection risks for displaced populations [1420][1412][1419]. The ICRC notes large-scale displacement and the need for unimpeded access to assist affected populations, reporting extensive assistance in 2025 and warning that millions remain displaced across Sudan [1486].

Gender based violence (GBV)

In early June 2026, Legal Action Worldwide filed a criminal complaint in Nairobi on behalf of 12 Sudanese victims seeking prosecution of 10 alleged RSF members for torture and sexual violence committed in and around Khartoum between April 2023 and March 2025, some of whom are believed to be residing in Kenya [1458][1435][1469][1464][1455]. The complaints allege rape, sexual slavery and other forms of conflict-related sexual violence, alongside beatings, burns, suffocation, electric shocks, prolonged inhumane detention with inadequate food and water, and forced movement of dead bodies from detention sites [1435][1458][1469][1464][1451]. The Kenya filing invokes domestic international-crimes mechanisms and places Kenya at the centre of a potential universal-jurisdiction effort, with prosecutors given a statutory window to decide whether to open an investigation [1469][1455][1435].

The UN Independent International Fact-Finding Mission concluded in February 2026 that RSF forces committed crimes against humanity, war crimes and genocide during the October 2025 siege and takeover of El Fasher, amid widespread reports of sexual and gender-based violence across Sudan [1481]. UN mandate holders and rights groups report that rape has been used publicly to terrorize populations and have documented pervasive conflict-related sexual violence, while international actors warn that impunity remains a central driver and are urging investigations and accountability measures [1453][1481][1461][1448]. Rapid assessments in West Kordofan (Ghubaysh) and East Darfur have additionally documented reports of SGBV on displacement routes and identified a lack of safe and dignified facilities, safe spaces for women and girls, and confidential case management as critical gaps requiring urgent attention [1420][1412][1417][1419].

Factors driving protection risks

Ongoing heavy fighting, aerial and drone strikes, and repeated targeting of markets, villages and health infrastructure have driven acute civilian harm across Sudan [1442][1441][1481]. External state sponsorship and illicit gold-financing have sustained and fragmented the war, making it resistant to resolution and prolonging civilian exposure to violence [1413]. Explosive ordnance contamination, including landmines and unexploded remnants of war, is restricting access to agricultural land, services and livelihoods and raising casualty risk for civilians [1407][1489].

The collapse of basic services — including imminent closure of dozens of IRC-supported health facilities, medicine shortages, halted water trucking in Central Darfur, and outbreaks of cholera and measles — has amplified protection vulnerabilities and negative coping [1483][1412][1419]. Economic deprivation, rising food and commodity prices, and disrupted markets are driving negative coping, indebtedness and displacement, increasing exposure to exploitation and gender-based violence [1483]. Large-scale displacement — with UNHCR reporting nearly 927,915 refugees in eastern Chad as of 17 May 2026, almost 1 million Sudanese recorded in refugee-like or asylum situations in 2025, and over 11 million people displaced within Sudan — has strained host services and protection capacity [1414][1446][1486]. IOM cross-border monitoring documents long processing times, bribery/extortion and insecurity at crossing points, contributing to family separation, loss of belongings and administrative barriers to protection for returnees and movers [1408].

Widespread reports of sexual and gender-based violence, child recruitment, arbitrary detention and enforced disappearances, combined with limited safe spaces, inadequate GBV services and weak accountability mechanisms, further compound protection risks across affected areas [1481][1412][1419][1404]. UNICEF highlights sustained high child protection needs and gaps in mental health and psychosocial support, case management and services for children with disabilities, exacerbating vulnerabilities among displaced and conflict-affected children [1428].

Impact on mental health

Sudanese civilians are experiencing high levels of trauma — including recurring nightmares, acute PTSD symptoms and pervasive psychological distress — with reports citing that more than 70% of displaced youth show moderate to severe psychological symptoms [1415]. The scale of displacement, exceeding 11 million people within Sudan, amplifies demand for psychosocial support and strains limited services [1486]. Cultural stigma and norms suppressing help-seeking are widespread, leading to underreporting and barriers to care in affected communities [1415].

Humanitarian access constraints — including attacks on bridges, roads and health facilities and ongoing hostilities — are hindering sustained delivery of mental health services and psychosocial support [1479][1442][1482]. UNICEF delivered integrated child-protection and MHPSS services to 129,767 people between March–April 2026, including 67,619 children and 62,148 adults, and offered community-based MHPSS to 17,274 children and 8,249 parents/caregivers; however, coverage overall remains limited [1428]. Assessments identify persistent gaps in specialized MHPSS, late identification of vulnerable children including those with disabilities, and a shortage of safe, confidential spaces and psychological services for women and girls [1428][1412][1419]. Local practitioners and commentators urge rapid expansion of community-based safe spaces, telepsychological hotlines, and integrated MHPSS interventions to reach displaced youth and survivors of violence as response priorities [1415][1428][1412].

Protection needs and priorities

Rapid multi-sector assessments in West Kordofan (Ghubaysh), conducted 19–25 May 2026, documented severe protection gaps — including reports of SGBV on displacement routes, lack of safe and dignified facilities or safe spaces for women and girls, absence of confidential case management and weak accountability — and recommended establishing safe spaces, confidential case management and psychosocial support, integrating GBV risk mitigation measures such as lighting and gender-segregated WASH, and conducting awareness sessions on available services [1420][1412].

An assessment in Khazan Jedeed (East Darfur) in early May similarly identified major protection concerns including family separation, unaccompanied and separated children, and cases of sexual violence, and called for scaled-up community-based MHPSS, outreach and strengthened referral pathways [1417][1419].

UNICEF reporting from March–April 2026 shows substantial child protection delivery — reaching 129,767 people with MHPSS, case management and related services — but highlights persistent gaps in specialized services, late identification of vulnerable children and weaknesses in referral pathways, indicating limits to the effectiveness of current protection monitoring and case management systems [1428]. IOM reported continued coordination of the returnee response in eastern Chad and assistance to protection cases, including 55 people with specific needs in Ouaddaï, 15 women at risk in Sila and 14 protection cases in Wadi Fira, while tracking large returnee flows — with 400,914 Chadian returnees registered as of 31 May 2026 — that increase protection pressures on host areas [1433]. A joint mission including UNFPA and OCHA visited Ngoubi (18 km from Bria) on 3 June and registered 31 households (130 people) among CAR returnees from Sudan, identifying protection as a priority need alongside food, shelter, WASH and health services [1429].

Operational Environment

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